

Sleep, Dreams and Fluvoxamine

A Complete Plain-Language Reference

Sleep stages and cycle architecture • REM and dreaming • vivid dreams and unrefreshing mornings • daytime sleepiness and naps • sleep apnea and narcolepsy • Obsace ER (fluvoxamine) in full detail

Who this is written for

This assumes no medical background at all. Every technical term is explained where it first appears, and there is a full glossary at the end. Where a detail is easy to miss, it is called out rather than left implied.

It is designed to be read straight through once, then used as a reference — the contents page and glossary are there for looking things up later.

Important — please read before anything else

This document explains general medical science. It is **not a diagnosis, not a treatment plan**, and cannot replace the doctors who know your history.

Do not use it to start, stop, split, skip or change the dose of any prescribed medicine. Antidepressants in particular require supervised changes; stopping suddenly causes withdrawal symptoms and risks the return of the condition being treated.

The right way to use this: read it, mark what sounds like you, and take those specific points and questions to your psychiatrist and sleep physician.

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Glossary

PART 1

How Sleep Is Built

1.1 Sleep is a sequence, not a switch

Most people picture sleep as a switch: awake, then off, then awake again. That picture is wrong, and almost every confusing thing about sleep becomes clear once it is replaced.

Sleep is a **sequence of several distinct states**, each doing a different job, running through the night in a repeating pattern. Think of it as a night shift in a factory: different teams clock in at different hours, each doing work the others cannot do. If one team never gets its shift, the factory opens in the morning with part of the work undone — even though the building was shut all night.

That is the central idea of this whole document. **Time in bed is not the same as work completed.** You can spend eight hours asleep and wake with the job half finished.

1.2 The two families: NREM and REM

- **NREM sleep** — Non-Rapid Eye Movement. The quiet, physically restorative family. Three stages: N1, N2, N3. Roughly **75–80%** of a normal adult night.
- **REM sleep** — Rapid Eye Movement. The active, dreaming family. The brain is nearly as busy as when awake, while the body is deliberately paralysed. Roughly **20–25%** of the night.

An older naming system called N3 "stages 3 and 4" and used the term stage S. Modern sleep reports use N1, N2, N3 and R. If an older doctor or an older report uses stage 4, it means the deepest part of what is now called N3.

1.3 Stage N1 — the doorway

Duration: 1–7 minutes per cycle | **Share of night:** ~5% | **Brain waves:** theta (4–8 Hz), replacing waking alpha

The thin border between waking and sleeping. Muscles loosen, thoughts drift into disconnected images, breathing slows and becomes regular. Slow rolling eye movements appear — these are what a sleep technician looks for to mark sleep onset.

Woken from N1, most people insist they were never asleep, and they are half right: N1 is a genuine in-between state. This is also where **hypnic jerks** occur — the sudden kick of a limb, sometimes with a sensation of falling or a flash of light or sound. It is a normal handover glitch as motor control is released, experienced by around 70% of people at some point.

N1 rises sharply in people with fragmented sleep, because every arousal drops them back to the doorway and they must descend again. A sleep report showing a high percentage of N1 is a strong objective sign of a disturbed night.

1.4 Stage N2 — the workhorse

Duration: 10–25 min early, lengthening later | **Share of night:** 45–55% — the largest | **Brain waves:** theta with sleep spindles and K-complexes

Roughly half of your sleeping life is spent here. Body temperature falls, heart rate slows, muscle activity drops further. Two distinctive electrical signatures define it:

- **Sleep spindles** — short bursts of fast activity (12–16 Hz) lasting half a second to two seconds. They are the brain filing the day's learning into long-term storage, and they also act as a gate that blocks outside noise from reaching consciousness. Part of what "being a light sleeper" means is weaker spindle activity.

- **K-complexes** — single large sharp waves, often triggered by a sound or a touch. They appear to be the brain suppressing an arousal: registering a stimulus and deciding not to wake up.

N2 is genuine, useful sleep — not a waiting room. Memory consolidation, motor-skill learning and the protective gating of the sleeping brain all happen here.

1.5 Stage N3 — deep sleep, the repair shift

Duration: 20–40 min in early cycles, near zero by morning | **Share of night:** 15–25% | **Brain waves:** delta — large slow waves under 4 Hz

Also called slow-wave sleep or deep sleep. The hardest stage to be woken from: shake someone out of N3 and they are confused, slow and disoriented for several minutes. This is the stage that produces the feeling of being *physically* rested. During N3:

- **Growth hormone** is released in its largest pulse of the 24-hour day, driving tissue repair and muscle recovery.
- The **immune system** performs much of its maintenance; this is part of why sleep loss increases susceptibility to infection.
- The **glymphatic system** — the brain's waste-clearance network — runs at its highest rate, flushing metabolic by-products that accumulate during waking hours. The spaces between brain cells expand during deep sleep, allowing fluid to wash through more freely.
- **Testosterone** production, which is concentrated during sleep, depends heavily on obtaining sufficient deep sleep. Restricting sleep measurably lowers it within about a week.
- **Blood pressure and heart rate** reach their lowest point of the 24-hour cycle — the nightly "dip" that protects the cardiovascular system over decades.

N3 is front-loaded — remember this

The large majority of deep sleep happens in the **first half of the night**.

A person who sleeps 11pm–3am solidly and then has a broken second half still gets most of their physical restoration. Someone whose *first* three hours are disturbed loses far more than the clock suggests.

This also means going to bed much later than usual costs you deep sleep, while waking earlier than usual costs you REM. The two kinds of sleep loss are not interchangeable.

1.6 REM sleep — the dreaming shift

Duration: 5–10 min in cycle 1, up to 45–60 min in the last | **Share of night:** 20–25% | **Brain waves:** fast, low-amplitude, close to waking; distinctive sawtooth waves

REM is the strangest state the human body produces. Measured at the scalp the brain looks almost awake. The eyes dart rapidly behind closed lids. Breathing and heart rate become irregular. Temperature regulation largely switches off — during REM you neither sweat nor shiver effectively, which is why room temperature matters more than people realise. And the body cannot move at all.

REM atonia — why you cannot move

During REM the brainstem actively switches off signals to the voluntary muscles. You are temporarily paralysed from the neck down, apart from the eyes and the diaphragm.

This is protective. The brain is generating a full experience — running, fighting, falling — and without atonia you would act it out and injure yourself.

Two familiar experiences come from this system misfiring. **Sleep paralysis** is waking while atonia is still on: the mind is awake, the body will not move, and it can be frightening. **REM sleep behaviour disorder** is the reverse — atonia fails and the person physically acts out dreams.

REM is further divided into two sub-types, which matters because they feel different:

- **Tonic REM** — the steady background state: atonia present, brain active, no eye movement bursts.
- **Phasic REM** — bursts of rapid eye movements, muscle twitches (fingers, face), irregular breathing and heart rate. The most intense dreaming and the highest arousal threshold occur here.

What REM does:

- **Emotional processing.** REM appears to strip the emotional charge from difficult memories while preserving the memory itself — sometimes described as overnight therapy. Noradrenaline, the brain's stress chemical, falls to its lowest level of the 24-hour cycle during REM, which may be why difficult material can be reprocessed safely.
- **Associative and creative memory.** NREM files facts; REM connects them to each other, which is why solutions sometimes arrive after sleeping on a problem.
- **Nocturnal erections.** In men, erections occur automatically during REM — typically three to five per night, totalling over an hour. Morning erections are simply waking at the end of a REM period. Discussed fully in section 4.7.

1.7 Brain waves: what is actually being measured

A sleep study reads electrical activity from the scalp. Different states produce different wave frequencies, and this is how stages are scored:

Wave	Frequency	When it appears
Beta	13–30 Hz	Awake, alert, thinking
Alpha	8–12 Hz	Awake but relaxed with eyes closed; disappears at sleep onset
Theta	4–8 Hz	N1 and N2 — light and intermediate sleep
Delta	under 4 Hz	N3 — deep slow-wave sleep. Large, slow, high-amplitude
Spindles	12–16 Hz bursts	N2 — memory filing and noise gating
Sawtooth	2–6 Hz	REM — distinctive jagged waves, often just before eye-movement bursts

PART 2

The Cycle — How the Stages Actually Run

This is the part that most short explanations get wrong, and it is worth getting exactly right, because several confusing symptoms make sense only once the real ordering is clear.

2.1 The real order within a cycle

The common simplification is "N1 → N2 → N3 → REM, then repeat." That is not what happens. You do not go from deep sleep straight into REM. You go **down** into deep sleep and then **back up** through lighter sleep before REM begins.

The actual sequence of one cycle

Wake → N1 → N2 → N3 → back to N2 → REM → brief arousal → N2 → N3 → back to N2 → REM → ...

Read that carefully. After deep sleep you **ascend again through N2**, and only then enter REM. REM is not reached from the bottom of the descent; it is reached on the way back up.

And after REM ends you do not wake up and start fresh — you drop back into NREM, usually N2, and begin the next descent. **NREM → REM → NREM → REM** is the true alternating pattern, running continuously all night.

2.2 The descending and ascending halves

It helps to picture each cycle as a valley. You walk down one side, reach the floor, and walk back up the other side.

- **Descending limb:** N1 → N2 → N3. Sleep deepens. Arousal threshold rises — it takes progressively more noise to wake you. Body systems slow down.
- **The floor:** N3, the deepest point. Slow delta waves, maximum physical restoration.
- **Ascending limb:** N3 → N2 → REM. Sleep lightens again, passes back through N2, then switches into REM, which is electrically light but behaviourally deep — hard to wake from despite an active brain.

This asymmetry is why the timing of an alarm matters so much. An alarm landing on the descending limb or at the floor pulls you out of N3 and produces severe grogginess. An alarm landing on the ascending limb, in N2 or at the end of REM, produces a clean waking. Same amount of sleep, completely different morning.

2.3 Why you return to N2 before REM, every time

REM and deep sleep are produced by different, partly opposing brain systems. Deep sleep is driven by slow-wave-generating circuits; REM is switched on by a cluster of brainstem cells and switched off by another. The brain cannot run both states at once, and the transition is not instantaneous — it passes through the intermediate state, which is N2.

Practically, this means **N2 acts as the corridor between every room**. Every transition in either direction routes through it. That is a large part of why N2 occupies half the night: it is not only a destination, it is also the traffic between all the other destinations.

2.4 The brief awakenings between cycles

At the end of each REM period there is normally a **brief surfacing** — sometimes to full wakefulness for a few seconds, sometimes just to N1. You may turn over, adjust the blanket, glance at the clock. Almost always, you will not remember it.

This is **completely normal and universal**. A healthy adult has somewhere between 10 and 20 brief arousals a night and recalls none of them. It is thought to be an evolutionary safety check — a periodic scan of the environment before committing to another cycle of unconsciousness.

Why this matters for your own worries about sleep

Two things follow, and both reassure people who think they sleep badly.

First: waking briefly in the night is not insomnia. Insomnia is difficulty *returning* to sleep, or distress about the waking — not the waking itself.

Second: because these arousals cluster at the ends of REM periods, and REM is where vivid dreams occur, the normal architecture of sleep naturally produces some dream recall. That is expected. It becomes a signal only when it is heavy and constant, because then it means the arousals have become far more frequent than the normal 10 to 20.

2.5 How the cycles change across the night

A full cycle takes roughly **90 minutes** — realistically anywhere from 70 to 120, and it varies between cycles in the same person on the same night. A normal night contains four to six.

But the cycles are not copies of each other. The mix shifts steadily:

Cycle	Roughly when	Deep sleep (N3)	REM	Character of that cycle
1st	0–90 min	20–40 min — large	5–10 min — minimal	Physical repair dominates
2nd	90–180 min	Still substantial	15–20 min	Balanced
3rd	3–4.5 hr	Shrinking quickly	20–30 min	REM taking over
4th	4.5–6 hr	Often almost none	30–45 min	Mostly N2 and REM
5th / 6th	Final hours	Essentially none	Up to 60 min	REM-dominated; longest dreams

The single most useful fact in this guide

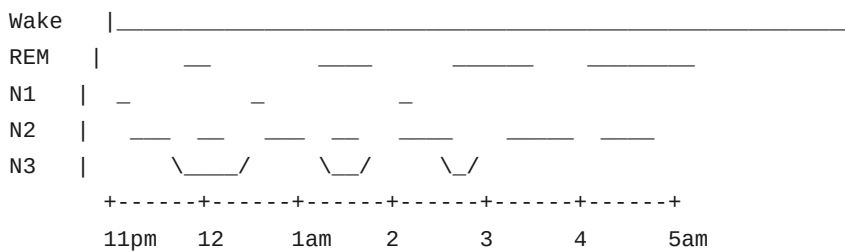
Deep sleep dominates the first half of the night. REM dominates the second half.

Consequences that all follow from this one asymmetry:

- Cutting sleep short by two hours does not remove two hours evenly — it removes almost entirely REM, because REM is what was scheduled for those final hours.
- The dreams you remember are usually the last ones, because the final REM period is the longest and is closest to waking.
- A drug that suppresses REM changes the shape of the second half of the night far more than the first.
- Going to bed very late costs you deep sleep. Waking very early costs you REM. Both are "sleep loss" but they damage different things.

2.6 Reading a hypnogram

A sleep study produces a **hypnogram** — a graph with stages on the vertical axis and time on the horizontal. It is the single most informative page in a sleep report, and it is readable once you know the shape. A normal night looks roughly like this, with deep troughs early and long REM blocks late:



What to look for on your own hypnogram when the report arrives:

- **Are there deep N3 troughs early?** If N3 is absent or shallow throughout, restoration is not happening.
- **Do REM blocks lengthen towards morning?** If REM appears very early or is scattered evenly, that is abnormal and is one of the patterns seen in narcolepsy.
- **How many wake lines break the picture?** A hypnogram that looks like a comb rather than a wave is a fragmented night.

2.7 A worked example of one real night

Someone in bed at 11:00pm, up at 6:30am — seven and a half hours:

Time	What is happening
11:00–11:12	Lying awake, drifting. This is sleep onset latency — normal is 10 to 20 minutes.
11:12–11:17	N1. Slow eye rolls, drifting images. A hypnic jerk is possible here.
11:17–11:35	N2. Spindles begin. Temperature dropping.
11:35–12:10	N3 — the deepest trough of the night. Growth hormone pulse. Hardest to wake.
12:10–12:20	Ascending back through N2.
12:20–12:28	First REM period — short. A dream occurs; almost certainly not remembered.
12:28–12:30	Brief arousal. Turns over. No memory of it.
12:30–1:55	Cycle 2. Still good N3, then N2, then ~18 minutes of REM.
1:55–3:25	Cycle 3. N3 shrinking noticeably. REM around 25 minutes.
3:25–4:55	Cycle 4. Very little N3. Mostly N2, then 35–40 minutes of REM.
4:55–6:30	Cycle 5. Essentially no N3. Long REM blocks, brief N2 between them.
6:30	Alarm. If it lands in REM or N2 — clean waking, dream possibly remembered. If sleep was disturbed and the brain was catching up on N3 — heavy grogginess.

Notice how the last three hours are almost entirely N2 and REM. That is why morning dreams are the vivid, memorable, story-like ones, and why anything affecting REM has its largest effect on how your morning feels.

PART 3

What Controls Sleep — The Two-Process Model

Two independent systems decide when you sleep and how deeply. Almost every sleep problem is a fault in one of them, or a conflict between them.

3.1 Process S: sleep pressure and adenosine

Process S is the homeostatic sleep drive — simply, the longer you are awake, the sleepier you become. It builds steadily from the moment you wake and discharges during sleep, most rapidly during deep N3 sleep.

The chemistry behind it: as brain cells consume energy through the day, a by-product called **adenosine** accumulates. Adenosine binds to receptors that suppress alertness. By evening, high adenosine equals strong pressure to sleep. During sleep it is cleared, and you wake with the gauge reset.

This is exactly how caffeine works

Caffeine does not give you energy. It has no fuel in it.

Caffeine fits into the same receptors adenosine uses, and blocks them without activating them. The adenosine is still there and still building — you simply stop feeling it.

This explains the crash. When the caffeine clears, all the adenosine that accumulated in the meantime binds at once, and the tiredness arrives suddenly and heavily.

It also explains why late caffeine damages sleep quality even when you fall asleep normally: blocked adenosine receptors mean a weaker drive into deep N3 sleep. You sleep, but more lightly. **Section 8.7 explains why this matters far more than usual for anyone taking fluvoxamine.**

3.2 Process C: the body clock

Process C is the circadian rhythm — an internal clock running on a cycle of roughly 24 hours (24.2 on average, which is why it needs resetting daily). It is generated by a cluster of about 20,000 cells in the hypothalamus called the **suprachiasmatic nucleus (SCN)**.

The SCN does not respond to tiredness. It responds mainly to **light**, detected by specialised cells in the retina that exist for this purpose rather than for vision. It then drives a daily rhythm in body temperature, hormone release, digestion, blood pressure, alertness and much else.

The crucial point: Process C decides *when* you should sleep, entirely independently of how tired you are. You can be exhausted and still lie awake because your clock says it is daytime. This is the whole of jet lag, and much of shift-work difficulty.

3.3 How the two interact — and the afternoon dip

Good sleep happens when both processes align: high sleep pressure (Process S) meeting the circadian window for sleep (Process C).

Through a normal day, Process C produces an **alerting signal** that rises through the day, deliberately counteracting rising sleep pressure so you do not become progressively more useless. It peaks in the early evening, then falls sharply — which is why sleepiness can arrive quite suddenly at night.

Why the afternoon dip exists

The circadian alerting signal does not rise smoothly. It has a **dip in the early-to-mid afternoon**, roughly 1pm to 4pm for most people.

At that moment, sleep pressure has been building for seven or eight hours while the alerting signal briefly weakens. The result is a genuine, biologically driven window of sleepiness.

This is not caused by lunch. It happens in people who skip lunch entirely. It is a real second sleep window built into human biology, and it is the reason siesta cultures exist.

It also explains why an afternoon nap can feel unusually efficient and easy — you are sleeping with the clock rather than against it.

3.4 Melatonin, light and core body temperature

Melatonin is the clock's chemical messenger. The SCN triggers the pineal gland to release it roughly one to three hours before your natural bedtime — an event called **dim light melatonin onset**, the most reliable marker of where your body clock actually sits.

Melatonin does not sedate you. It is a timing signal, not a sleeping drug — it tells the body that biological night has begun. Light, particularly blue-enriched light, suppresses its release directly. This is the mechanism behind screen advice: it is not eye strain, it is a hormone being actively switched off at the wrong hour.

Core body temperature runs on the same clock. It peaks in the early evening and falls through the night, reaching its minimum roughly two hours before habitual wake time. The *falling* temperature is itself a sleep trigger, which is why a warm bath or shower one to two hours before bed helps: blood moves to the skin, heat is dumped, and core temperature drops faster than it otherwise would. A cool bedroom works for the same reason.

Timing of light matters in a specific direction, and getting this backwards makes things worse:

- **Light in the morning** shifts your clock **earlier** — you become sleepy earlier and wake earlier. This is the fix for a clock that has drifted late.
- **Light in the late evening** shifts your clock **later** — pushing sleep onset further back and making mornings harder.

3.5 Chronotype, social jetlag and shift work

Chronotype is where your personal clock naturally sits. It is substantially genetic and it shifts with age — latest in the late teens and early twenties, drifting earlier from the mid-twenties onward. A late chronotype is not laziness and cannot be fixed by willpower; it can only be shifted, gradually, with light timing and consistent schedules.

Social jetlag is the mismatch between your biological clock and your imposed schedule. Someone whose body wants to sleep 1am–9am but who must wake at 6am is living with a permanent three-hour time-zone shift, without ever travelling. The typical pattern is unrefreshing early starts, sleeping much later on days off, and chronic daytime fatigue.

Shift work is the extreme version. Rotating shifts in particular repeatedly break the clock's attempts to settle, because it can only shift by about an hour per day. Two points that surprise people:

- **The effects can outlast the shifts by years.** A clock that has been disrupted for a long period does not automatically snap back when the schedule normalises. It can remain misaligned, and the

symptoms then get wrongly attributed to something else.

- **It masks other conditions.** Someone with genuine sleep apnea or a hypersomnia who is also doing rotating shifts will naturally blame the shifts, and may not investigate for years. When the shifts stop and the symptoms do not, that is important information — it means something else was always there.

3.6 Sleep debt: what can and cannot be repaid

Sleep loss accumulates. Six hours a night for a week is not a new normal you have adapted to — it is a debt, and objective performance testing shows people underestimate their own impairment badly. Subjective sleepiness plateaus while actual performance keeps declining.

Recovery is real but partial and uneven:

- **Deep sleep repays fastest.** After deprivation the first recovery night contains a large N3 rebound — the brain prioritises physical restoration.
- **REM repays second,** often on the second and third recovery nights, producing the flood of intense dreams people notice after a hard week. This is REM rebound (section 4.4).
- **Weekend catch-up helps but does not fully clear it,** and sleeping several hours later at weekends pushes the clock later, making Monday worse. The trade is rarely worth it.

Dreams

Dreaming and *remembering* a dream are two entirely different things. Confusing them is the source of nearly every misunderstanding people have about their own dreams — including the belief that they are dreaming too much.

4.1 REM dreams versus NREM dreams

Dreaming occurs in both families of sleep, but the experiences are not alike:

	REM dreams	NREM dreams
Quality	Vivid, visual, richly detailed	Thin, thought-like, often just an image
Structure	Story-shaped, with scenes and characters	Fragmentary, static
Emotion	Strong — fear, joy, anxiety	Muted
Logic	Bizarre; impossible things accepted	More plausible, mundane
Recall	Good if woken from it	Poor; often reported as "just thinking"
Example	Being chased through a building that is also your school	"I was thinking about work"

4.2 Why REM dreams are bizarre

During REM the brain is not uniformly active. Some regions run hot and others go quiet, and the specific pattern explains the character of dreams almost perfectly:

- **Visual association areas: highly active** — hence vivid imagery.
- **Amygdala and emotional centres: highly active** — hence the strong emotional charge, and why fear and anxiety are so common in dreams.
- **Hippocampus: active** — hence real memories, real people and real places appearing, recombined.
- **Dorsolateral prefrontal cortex: largely offline.** This is the region responsible for logic, sequence, self-monitoring and asking "wait, does this make sense?"

That last one is the key. The critic is asleep while the storyteller works at full power. It is why you can accept, entirely without surprise, that your childhood home is also your office and that a dead relative is present — and why the absurdity only becomes obvious after waking, when the critic comes back online.

The same mechanism explains why dreams are so hard to hold onto. The regions that would normally narrate and encode an experience into durable memory are not fully running.

4.3 The rule of dream recall

You dream every single night, four to six times, whether you recall it or not. Nobody stops dreaming. What varies is recall, and recall depends overwhelmingly on one factor:

The rule

You remember a dream only if you wake during it, or within about two minutes of it ending.

Dream content sits in an extremely fragile short-term store. Pass smoothly from REM into the next NREM stage without surfacing, and the content is never transferred into long-term memory. By morning it is gone as completely as if it never occurred.

This leads to a conclusion that surprises almost everyone:

Remembering many vivid dreams usually does not mean more dreaming. It means more waking.

Someone reporting "I dream all night, every night, and they are so vivid" is very often describing a night that is being broken up repeatedly — dozens or hundreds of brief arousals they are completely unaware of, each one catching a dream in progress and stamping it into memory. The dreams were always there. The fragmentation is what is new.

Two smaller factors also affect recall: waking naturally at the end of a REM period produces better recall than an alarm mid-cycle, and simply paying attention to dreams increases recall, which is why keeping a dream diary appears to increase dreaming when it only increases noticing.

4.4 REM rebound

REM behaves like a debt. Deprive someone of it — through short sleep, alcohol, or certain medicines — and pressure accumulates. Remove the obstacle and the brain takes it back with interest: more REM than usual, entered faster than usual, and noticeably more intense.

This is **REM rebound**, and it explains several things people find alarming:

- The flood of strange dreams after several sleep-deprived nights.
- Intense dreaming when someone stops drinking alcohol, which suppresses REM heavily in the first half of the night.
- Vivid dreaming on antidepressants — covered in section 8.5 — where suppression and rebound can occur within the same night.
- Vivid dreaming during withdrawal from sedatives, often the most intense dreaming people ever report.
- **False positives on narcolepsy testing** if a REM-suppressing drug is stopped abruptly before an MSLT. See section 7.4 — this is a genuinely important practical point.

4.5 Everything that makes dreams more vivid

Cause	Mechanism
Fragmented sleep, any cause	More awakenings during REM, so far more dreams get encoded
Obstructive sleep apnea	Breathing pauses cause arousals, many out of REM
Antidepressants (SSRI, SNRI, TCA)	REM suppression followed by rebound; plus more arousals
Stopping an antidepressant	Strong REM rebound — a recognised withdrawal symptom
Alcohol	Suppresses REM early; heavy rebound in the second half of the night
Alcohol or sedative withdrawal	Large REM rebound
Sleeping later or longer than usual	More time spent in the REM-rich final hours
Fever and illness	Increased arousals; altered brain chemistry
Stress, grief, trauma	Greater emotional processing load plus more arousals
Beta blockers	Reduce melatonin and alter REM; nightmares are a known effect
Nicotine, and nicotine patches worn overnight	Stimulant effect plus REM changes; vivid dreams common
Narcolepsy	REM intrudes where it should not, including at sleep onset and into naps
Pregnancy	Hormonal change plus frequent awakenings
Late heavy meals	More nocturnal arousals

4.6 Nightmares, parasomnias and what they signal

Parasomnias are unwanted events during sleep. They split cleanly by which family of sleep they arise from, and the distinction is genuinely useful because it tells you which half of the night they belong to.

NREM parasomnias — from deep sleep, first half of the night

- Sleepwalking, sleep talking, confusional arousals, night terrors.
- The person is **not dreaming** and typically has no memory of the event at all. A night terror is not a nightmare: the person may scream and appear terrified while being essentially unconscious, and will recall nothing.
- Triggered by anything that deepens sleep or fragments it: sleep deprivation, fever, alcohol, stress, and obstructive sleep apnea.

REM parasomnias — from REM, second half of the night

- **Nightmares** — vivid, frightening dreams that wake you, with clear recall. Occasional ones are normal. Frequent ones warrant attention, particularly if they follow trauma or began after starting a medication.
- **Sleep paralysis** — waking with atonia still active. Frightening but harmless. More common with sleep deprivation and irregular schedules, and notably more common in narcolepsy.
- **REM sleep behaviour disorder** — atonia fails and the person physically acts out dreams, sometimes violently. **This one always needs medical assessment**, both for injury risk and because it can be an early marker of neurological conditions appearing years later.

4.7 Nocturnal erections and what they prove

In men, erections occur automatically during REM sleep — typically three to five per night, totalling an hour or more. They are entirely involuntary, require no sexual content in the dream, and occur from infancy to old age. The medical term is **nocturnal penile tumescence**.

Morning erections are not a separate phenomenon. They are simply the last of the night's REM erections, still present because you happened to wake at the end of a REM period — which, as Part 2 explained, is exactly when most natural waking occurs.

Why doctors ask about this

Nocturnal erections happen with **no conscious involvement whatsoever**. The mind is not participating, and neither anxiety nor expectation can interfere.

So their presence demonstrates that the nerves, the blood vessels and the hormonal support are all functioning. If erections occur reliably during sleep but fail when awake, the physical machinery is intact and the problem lies in what happens while conscious — which is a far more treatable situation.

Their reduction or absence can point the other way — towards hormonal, vascular or neurological causes. But there is an important catch, and it is easy to get wrong:

Anything that reduces REM sleep also reduces nocturnal erections, because they only occur during REM. That includes antidepressants, sleep apnea and any badly fragmented night. So reduced morning erections in someone on an SSRI, or with untreated apnea, do not automatically indicate a hormonal or vascular problem — they may simply reflect less REM sleep.

Vivid Dreams Plus No Morning Energy

5.1 The short answer

Heavy dream recall together with unrefreshing mornings is, in most cases, a description of fragmented sleep.

The dreams are remembered because the night is being broken up (section 4.3). The mornings are flat because the same fragmentation prevents the consolidated deep sleep that produces the feeling of restoration (section 1.5). One underlying problem, two visible symptoms — which is why they so often appear together.

The person has been in bed eight hours. The clock says they slept. But the sleep was shallow and repeatedly interrupted, so the restorative work never got finished.

5.2 Why fragmentation is invisible from the inside

An arousal does not need to be a full awakening. The brain can lift out of deep sleep into light sleep for three to fifteen seconds — enough to abort the restorative process — and then drop back down, leaving no memory at all. Memory formation requires a longer period of wakefulness, usually around one to two minutes.

Someone with moderate sleep apnea may have **two to four hundred** of these in a night and sincerely report sleeping straight through. They are not mistaken about their experience; they are simply unable to perceive it. This is the single strongest reason sleep problems require measuring rather than self-reporting.

A useful analogy: imagine being tapped on the shoulder every two minutes all night. Each tap is too brief to remember, but none of your deep work ever gets finished. In the morning you would report sleeping eight hours and feeling terrible, with no idea why.

5.3 The full list of causes

1. Medication effects

SSRIs, SNRIs and tricyclics suppress REM and increase arousals, producing precisely this combination. Beta blockers reduce melatonin and cause nightmares. Stimulants taken too late fragment sleep. Corticosteroids disturb it. Some blood pressure medicines and some asthma medicines do too. **Fluvoxamine specifically is covered in full in section 8.5.**

2. Obstructive sleep apnea

Throat soft tissue collapses during sleep, blocking airflow. Oxygen falls, the brain arouses briefly to reopen the airway, breathing resumes, sleep returns — repeating all night. Full detail in section 6.3.

3. Circadian misalignment

The sleep may be structurally normal but occurring at biologically wrong hours, with morning waking landing during the internal night. Years of rotating shift work is a common cause, and the misalignment can persist long after the shifts end (section 3.5).

4. Periodic limb movement disorder

Repetitive leg movements every 20 to 40 seconds through the night, each capable of causing a micro-arousal. The sleeper almost never knows; a bed partner usually does. Strongly associated with low iron and ferritin, and — importantly — **SSRIs can cause or worsen it**. Picked up automatically by an overnight sleep study.

5. Central hypersomnias, including narcolepsy

Here the sleepiness is primarily generated rather than being a consequence of poor-quality nights. Full detail in section 6.4. The nap pattern is what separates this from everything above.

6. Nutritional, metabolic and hormonal contributors

- **Low vitamin B12** — fatigue, poor concentration, low mood; linked to restless legs.
- **Low vitamin D** — associated with fatigue and poorer sleep quality.
- **Low iron or ferritin** — the strongest single link to restless legs and periodic limb movements.
- **Thyroid disorders** — both underactive and overactive disturb sleep and energy.
- **Low testosterone** — fatigue, low mood, reduced morning erections. Note the loop: poor sleep lowers testosterone, and low testosterone worsens sleep.
- **Blood sugar dysregulation** — nocturnal fluctuations can cause awakenings.

These are worth correcting regardless of what else is found, because they are simple, cheap, and they overlap with and worsen every other cause on this list.

7. Psychological and behavioural contributors

- **Rumination at bedtime** — lying awake replaying past events delays onset and increases arousals.
- **Anxiety** — raises physiological arousal, lightening sleep throughout the night.
- **Depression** — characteristically shortens the time to first REM, increases total REM, and causes early-morning waking. Notably, this is one reason antidepressants suppress REM: it may be part of how they work.
- **Long or late naps** — discharge the sleep pressure needed for a consolidated night, creating a self-sustaining loop of bad nights and needed naps.

5.4 Sleep inertia — the most underrated explanation

Sleep inertia is the grogginess immediately after waking. It is a genuine physiological state, not a mood: blood flow to parts of the brain — especially the prefrontal regions handling decision-making — takes time to return to waking levels. Measured cognitive performance during severe sleep inertia can be worse than after moderate alcohol intake.

It normally lasts 15 to 30 minutes. After disrupted sleep it can last an hour or more. Its severity depends almost entirely on **which stage you were in when you woke**:

Woken from	Resulting inertia
N1 or N2 (light sleep)	Minimal — alert within a few minutes
REM	Mild — often clear-headed, with dream recall
N3 (deep sleep)	Severe — heavy, confused, disoriented for 30–60 minutes

This is the key to one of the most common complaints in this whole document. If your night has been fragmented, your brain may still be catching up on deep sleep in the early morning — the deep sleep that should have happened at midnight is being attempted at 6am. Your alarm then pulls you directly out of

N3, producing severe inertia. A short afternoon nap, which never reaches N3, produces almost none. **The night gave you more sleep; the nap gave you a better exit.**

5.5 Why you cannot self-diagnose this

Every cause in section 5.3 produces broadly the same felt experience: tired mornings, poor-quality sleep, heavy dreaming, daytime fatigue. You cannot tell them apart from the inside, and neither can a doctor from the description alone.

That is not a failure of attention — it is the nature of the problem. The things that actually distinguish these causes are an AHI number, a count of arousals per hour, the percentage of the night spent in each stage, a mean sleep latency, a count of SOREMPs, a ferritin level, a testosterone level. Every one of them is invisible from the inside, and that is precisely why the tests exist.

Daytime Sleepiness and the Refreshing Nap

People often mention this almost apologetically, as if it were an embarrassing habit rather than a symptom. In fact it is one of the most diagnostically valuable things a person can report, because the different explanations point to very different places.

One distinction first, because doctors will ask and the words are not interchangeable:

- **Fatigue** — physical or mental exhaustion, low energy, but you could not actually fall asleep if you tried. Typical of anaemia, thyroid disease, depression, B12 deficiency.
- **Sleepiness** — an active drive to sleep; you would fall asleep if you sat still in a quiet room. Typical of sleep apnea, insufficient sleep, and the hypersomnias.

The two can coexist, but which one predominates changes the direction of investigation entirely.

6.1 Four explanations for the refreshing nap

Explanation 1: Sleep inertia — a better exit, not better sleep

Covered in 5.4. A short nap stays in N1 and N2 and never reaches deep sleep, so you wake cleanly. A full night ends in whichever stage your brain happens to be in when the alarm sounds — and after a disturbed night that is often N3. Same person, same physiology, totally different waking experience.

Explanation 2: The afternoon circadian dip

Covered in 3.3. Sleeping during the natural afternoon window works with the clock rather than against it, producing efficient sleep and clean waking.

Explanation 3: The nap is genuinely better sleep

If the underlying problem is worse at night — apnea aggravated by hours lying flat, or a body clock misaligned with the night-time hours — then a daytime nap may simply be higher quality. Less time to accumulate disruption, possibly a different sleeping position, different timing relative to the clock. Two unbroken hours can beat eight fragmented ones.

Explanation 4: Narcolepsy

The one that matters most to identify, because it is a specific diagnosis with specific and effective treatment. Detailed in 6.4.

6.2 Nap length and what each length does

Length	What happens	Result
10–20 min	Stays in N1 and N2. No deep sleep reached.	The power nap. Clean waking, real alertness boost lasting 1–3 hours. Best general-purpose nap.
30–60 min	Enters N3 but is interrupted mid-way.	Worst option. Wakes out of deep sleep — heavy grogginess that can last an hour.
90 min	One complete cycle including REM.	Wakes at a natural cycle end. Refreshing, and includes REM benefits. Good if time allows.
2–3 hours	Multiple cycles.	Restorative at the time, but discharges substantial sleep pressure — the following night will be harder.

Two practical rules follow. Keep naps **under 30 minutes** unless you can afford a full 90. And keep them **before 3pm**, because later naps remove the sleep pressure needed for a consolidated night and can start a self-sustaining cycle: bad night, needed nap, worse night.

The exception worth knowing: in narcolepsy, short scheduled naps are not a bad habit — they are part of the recommended treatment, because they genuinely restore function for a couple of hours.

6.3 Obstructive sleep apnea in detail

What happens mechanically: during sleep the muscles holding the upper airway open relax. In susceptible people the airway narrows or closes completely. Airflow stops despite continued effort to breathe. Blood oxygen falls. Carbon dioxide rises. The brain detects this and triggers a brief arousal, muscle tone returns, the airway reopens — often with a gasp or loud snort — and sleep resumes. Then it repeats.

An **apnea** is a complete stop of at least 10 seconds. A **hypopnea** is a partial reduction with an oxygen drop or an arousal. Both count towards the AHI.

Two distinct kinds of damage result: the **fragmentation** destroys sleep quality, and the repeated **oxygen drops** stress the cardiovascular system — which is why untreated apnea raises the risk of high blood pressure, heart disease, stroke, type 2 diabetes and accidents.

Typical signs:

- Loud habitual snoring, often with witnessed pauses then a gasp.
- Waking with a dry mouth, sore throat, or morning headache.
- Unrefreshing sleep despite adequate hours; heavy daytime sleepiness.
- Nocturia — waking repeatedly to urinate. Frequently overlooked and frequently misattributed to the prostate.
- Reduced concentration and memory; irritability.
- **Erectile dysfunction** — apnea is an independent cause, through oxygen drops, fragmented sleep, reduced REM and lowered testosterone.

Risk rises with excess weight, a larger neck circumference, male sex, increasing age, nasal obstruction, and certain jaw and airway shapes — including a narrow airway or recessed jaw, which is why apnea occurs in slim people and is then missed for years. Alcohol and sedatives worsen it by relaxing the airway further.

Central sleep apnea is a different condition sharing the name: here the brain briefly fails to send the signal to breathe, so there is no effort at all rather than a blocked effort. Less common, different causes, and the sleep study distinguishes them by measuring breathing effort alongside airflow.

Treatment is effective. **CPAP** — a mask delivering gently pressurised air that splints the airway open — is the standard and works well when tolerated. Alternatives include mandibular advancement devices, positional therapy where apnea occurs mainly on the back, weight reduction where relevant, treating nasal obstruction, and surgery in selected cases.

6.4 Narcolepsy in detail

Narcolepsy is widely misunderstood, largely because of how films portray it. It is not comic collapse into sudden sleep. It is a disorder of the brain's control over the **boundary** between sleeping and waking.

What is actually going wrong

In **type 1 narcolepsy**, the brain has lost the cells that produce **orexin** (also called hypocretin) — a chemical whose job is to hold the awake state stable. The loss is thought to be autoimmune in most cases, often following an infection, and typically permanent.

Without enough orexin the boundaries blur **in both directions**. Sleep leaks into the day, and wakefulness leaks into the night. Elements of REM — dreaming, muscle paralysis — appear while awake.

This two-way leak explains a picture that otherwise seems self-contradictory: **overwhelming daytime sleepiness together with a broken, restless night**. People are often surprised that a condition of excessive sleep involves sleeping badly, and it is one reason the diagnosis gets delayed for years.

Type 2 narcolepsy has the sleepiness and the REM abnormalities but no cataplexy, and orexin levels are usually normal.

The classic five features (rarely all present in one person):

- **Excessive daytime sleepiness** — always present; usually the first symptom to appear, often in the teens or twenties. Frequently mistaken for laziness, depression or poor sleep habits for years before diagnosis.
- **Cataplexy** — sudden loss of muscle tone while fully awake, triggered by strong emotion, classically laughter but also surprise, anger or excitement. It may be dramatic (collapsing) or very subtle — the jaw slackening, the head nodding, the knees briefly buckling, speech slurring for a moment. Consciousness is fully retained throughout, which distinguishes it from fainting or seizures. When clearly present it is close to diagnostic. **Mention it explicitly if it has ever happened**, because people rarely volunteer it — they assume it is unrelated.
- **Sleep paralysis** — more frequent than in the general population.
- **Hypnagogic hallucinations** — vivid dream imagery while falling asleep or waking, often frightening and realistic. Commonly mistaken for a psychiatric symptom, which sometimes leads to years of the wrong treatment.
- **Disrupted night-time sleep** — frequent awakenings, despite the daytime sleepiness.

Additional features that support the picture: refreshing short naps; dreaming during very brief naps (normally impossible, since REM takes 70–100 minutes to arrive); automatic behaviour, where the person continues an activity while partly asleep and has no memory of it; and falling asleep within minutes of lying down at any time of day.

Treatment is symptomatic but often effective: scheduled short naps as a core strategy, wake-promoting agents such as modafinil or armodafinil, sodium oxybate for night-time fragmentation and cataplexy, and — notably — antidepressants are sometimes used specifically *because* they suppress REM, which helps cataplexy.

6.5 Idiopathic hypersomnia and the other look-alikes

Idiopathic hypersomnia is the main alternative diagnosis and is important to distinguish, because its pattern is almost the mirror image of narcolepsy in one key respect:

Feature	Narcolepsy	Idiopathic hypersomnia
Naps	Short and refreshing	Long and not refreshing
Night sleep	Fragmented	Often long and deep
Waking	Variable	Severe sleep inertia — "sleep drunkenness"
REM on MSLT	Two or more SOREMPs	Fewer than two SOREMPs
Cataplexy	In type 1	Absent

Other causes of genuine daytime sleepiness that must be excluded first: chronic insufficient sleep (by far the commonest), sleep apnea, circadian rhythm disorders, periodic limb movement disorder, medication effects, depression, thyroid disease, anaemia, and B12 deficiency.

6.6 The decisive question: does fixing the night fix the day?

The most useful single observation a patient can make

If night-time sleep quality improves and daytime sleepiness does not improve with it, that is significant.

When sleepiness is *caused* by poor-quality sleep — apnea, fragmentation, medication effects — improving the nights should improve the days. The daytime symptom is downstream of the night-time one.

When the two move independently, it suggests the daytime sleep drive is being generated in its own right rather than as a consequence. That is the pattern seen in the central hypersomnias, including narcolepsy.

This is why keeping a written record over weeks is worth so much more than a general impression. Tracking night quality and day sleepiness as two separate numbers reveals whether they move together or apart — and that single observation genuinely changes which diagnosis is most likely.

The Tests

7.1 Polysomnography — what is measured

The overnight sleep study. You sleep in a monitored room with sensors attached — uncomfortable at first, but most people sleep adequately, and the study accounts for a somewhat worse night than usual.

What is recorded simultaneously:

- **EEG** — brain waves, to score the stages (Part 1.7).
- **EOG** — eye movements, to identify REM and sleep onset.
- **EMG** — muscle tone at the chin (confirms REM atonia) and at the legs (detects periodic limb movements).
- **Airflow** at nose and mouth — detects apneas and hypopneas.
- **Respiratory effort** belts on chest and abdomen — distinguishes obstructive from central apnea.
- **Pulse oximetry** — blood oxygen saturation through the night.
- **ECG** — heart rate and rhythm.
- **Body position** — shows whether events cluster when lying on the back.
- **Video and sound** — snoring, movements, parasomnias.

7.2 The numbers in your report and what they mean

Measure	What it is	Normal range
Total sleep time	Actual sleep, not time in bed	7–9 hours for adults
Sleep onset latency	Minutes from lights out to sleep	10–20 min. Under 5 suggests sleepiness or deprivation
Sleep efficiency	Percentage of time in bed actually asleep	Above 85%
WASO	Wake After Sleep Onset — minutes awake during the night	Under 30 min in younger adults; rises with age
N1 %	Light transitional sleep	Under 5%. High N1 indicates fragmentation
N2 %	Intermediate sleep	45–55%
N3 %	Deep sleep	15–25%. Declines naturally with age
REM %	Dreaming sleep	20–25%
REM latency	Minutes from sleep onset to first REM	70–100 min. Short latency is significant
Arousal index	Brief arousals per hour	Under 15 per hour
AHI	Apneas + hypopneas per hour	Under 5 normal; 5–15 mild; 15–30 moderate; over 30 severe
Oxygen nadir	Lowest oxygen saturation recorded	Above 90%
PLM index	Periodic limb movements per hour	Under 15, and significant mainly if causing arousals

7.3 MSLT — how narcolepsy is actually diagnosed

The **Multiple Sleep Latency Test** runs the day after the overnight study. You are given four or five scheduled 20-minute nap opportunities, roughly two hours apart, in a dark quiet room, with sensors still attached. Between naps you must stay awake.

Two things are measured:

- **Mean sleep latency** — the average minutes taken to fall asleep across all naps. **8 minutes or less** indicates pathological daytime sleepiness.
- **SOREMPs** — Sleep Onset REM Periods: naps in which REM appears within 15 minutes of falling asleep. Normally REM should not appear in a short nap at all, since it takes 70 to 100 minutes to arrive. **Two or more SOREMPs together with a mean latency of 8 minutes or less is the core diagnostic finding for narcolepsy.** A SOREMP on the preceding overnight study can count as one of the two.

This is why the overnight study must come first: it confirms you slept adequately, so the next day's sleepiness cannot be explained away as simple sleep deprivation, and it rules out apnea as the cause.

Conditions required for a valid test, which is why the preparation instructions matter:

- Adequate sleep for at least one to two weeks beforehand — often documented with a sleep diary or an actigraphy watch.
- A regular sleep schedule in the run-up.
- No caffeine on the test day.
- A urine drug screen is commonly taken on the day.
- REM-suppressing and stimulant medications managed in advance — see below.

7.4 Why medication timing can invalidate the MSLT

This is the most important practical point in Part 7

The MSLT diagnoses narcolepsy by detecting REM where it does not belong. **Antidepressants suppress REM** — SSRIs, SNRIs and tricyclics all do, and it is one of their most consistent effects on sleep.

False negative risk: if REM is being pharmacologically suppressed on the day of testing, SOREMPs may not appear even in someone who genuinely has narcolepsy. The test comes back normal and the diagnosis is missed.

False positive risk: if a REM-suppressing drug is **stopped abruptly** shortly before the test, REM rebound (section 4.4) can create SOREMPs in someone who does not have narcolepsy.

Both directions of error are real. This is why standard protocols generally require REM-suppressing medication to be withdrawn **gradually, around two weeks before testing** — and why doing it suddenly is worse than not doing it at all.

But: whether stopping is safe depends entirely on the psychiatric condition being treated. For some people it is straightforward; for others the risk of relapse outweighs the diagnostic benefit, and the sleep physician will interpret the results in light of the medication instead. **This is a decision for the treating doctors together, never for the patient alone.**

What you can do: make certain the sleep physician has your **complete** medication list, including supplements, and ask directly whether anything needs to be adjusted before testing.

Drugs that affect sleep testing:

Drug class	Effect on REM	Typical washout before MSLT
SSRIs	Strong suppression	About 2 weeks (fluoxetine longer — it clears slowly)
SNRIs	Strong suppression	About 2 weeks
Tricyclics	Very strong suppression	About 2 weeks
Bupropion	Less REM suppression than SSRIs; may increase REM	Discuss individually
Mirtazapine	Suppression plus sedation	About 2 weeks
Stimulants and modafinil	Mask sleepiness entirely	Usually 1–2 weeks
Benzodiazepines, Z-drugs	Reduce deep sleep; alter latency	Usually 1–2 weeks
Sedating antihistamines	Cause drowsiness — confound results	Several days

Do not act on this table. It is here so the conversation with your doctors makes sense — the actual decision is individual.

7.5 Epworth score and sleep diaries

The **Epworth Sleepiness Scale** is a short questionnaire rating how likely you are to doze in eight everyday situations — reading, watching television, sitting in a car as a passenger, and so on — each scored 0 to 3. Total out of 24. Roughly: 0–10 normal, 11–14 mild, 15–17 moderate, 18+ severe excessive sleepiness. It is subjective and easy to under-report, but it gives a number that can be tracked over time.

A **sleep diary** kept for two weeks is often more informative than anything you can remember, and costs nothing. Template in section 9.3.

Fluvoxamine (Obsace ER) in Full Detail

8.1 What it is and what it treats

Obsace ER is a brand of **fluvoxamine**, an **SSRI** — Selective Serotonin Reuptake Inhibitor. It was the first SSRI approved for obsessive-compulsive disorder, and that remains its most characteristic use. In India it is prescribed most often for OCD and intrusive thoughts, also for anxiety disorders, social anxiety, panic disorder and depression.

Worth establishing: what is it treating in your case?

This single question determines almost everything else — what dose is appropriate, how long it should take to work, what counts as success, and which alternatives even exist.

The alternatives to an SSRI for depression are numerous. The alternatives for OCD are very few, because most non-SSRI antidepressants do not work well for it. So "can I switch to something with fewer side effects?" has a very different answer depending on the indication.

If you do not know which it is, ask. It is a reasonable question and the answer changes the whole conversation.

8.2 How it works

Nerve cells communicate by releasing chemical messengers into the gap between them, called the synapse. Serotonin is one such messenger. After delivering its signal, the releasing cell pulls it back in and recycles it — this is **reuptake**.

Fluvoxamine blocks that recycling pump. Serotonin therefore remains in the synapse longer and continues signalling. But the raised serotonin is not itself the treatment — that happens within hours, while the benefit takes weeks. Over those weeks the brain **adapts**: receptors adjust their sensitivity and downstream systems recalibrate, and it is this adaptation that produces the therapeutic effect.

This explains several things people find confusing: why it takes four to six weeks to work; why side effects often appear immediately while benefit does not; why anxiety can briefly worsen in the first week or two; and why stopping suddenly causes problems — the brain has adapted to the drug's presence and needs time to adapt back.

Fluvoxamine has one additional property most SSRIs lack: it is a potent **sigma-1 receptor agonist**. This receptor is involved in stress response and cellular regulation, and it may contribute to its effects in OCD and anxiety. It is a genuine pharmacological difference, not a marketing claim.

8.3 Extended release versus immediate release

	Immediate release	Extended release (ER)
Dosing	Often twice daily above 100 mg	Once daily
Release	Absorbed quickly	Released gradually over hours
Peak levels	Sharper peak	Smoother, lower peak
Side effects	More nausea and sedation peaks	Generally better tolerated
Timing	Usually evening	Usually bedtime

Practical point: extended-release tablets must be swallowed whole. Crushing, splitting or chewing destroys the release mechanism and delivers the entire dose at once.

8.4 Dosing, timing and how long it takes to work

	Fluvoxamine ER
Usual starting dose	100 mg once daily at bedtime
Usual therapeutic range	100 – 300 mg daily
Maximum	300 mg daily
Typical dose increment	50–100 mg, no more often than weekly
Typical OCD dose	Often towards the higher end of the range
Half-life	Roughly 15–20 hours
Time to steady state	About 1 week after any dose change
Time to benefit — mood/anxiety	4–6 weeks at an adequate dose
Time to benefit — OCD	10–12 weeks; sometimes longer

So a dose of 100 mg sits at the **lower end** of the range. That is neither good nor bad on its own — it depends entirely on what is being treated and how well it is working. It simply means there is room to move if the prescriber judges that appropriate.

Why bedtime dosing: fluvoxamine is among the more sedating SSRIs, so night-time dosing turns drowsiness from a problem into a benefit. The trade-off is that its effects on sleep architecture land squarely on your sleeping hours — the subject of the next section.

8.5 Effects on sleep architecture

This is the section that connects everything in Parts 1 to 6. SSRIs change the shape of sleep in consistent, well-documented ways:

Effect	What happens physiologically	What the person notices
REM suppression	Total REM time falls; REM latency lengthens	Fewer dreams at first; less emotional processing
REM rebound	Suppressed REM returns with greater intensity, sometimes within the same night	Unusually vivid, strange, memorable dreams
Increased arousals	More brief awakenings through the night	Sleep feels shallow; far more dreams recalled
Reduced sleep efficiency	Less of the time in bed is actual sleep	Eight hours in bed feels like five
Reduced deep sleep	N3 slow-wave sleep can decrease	Waking unrefreshed despite adequate hours
Periodic limb movements	SSRIs can cause or worsen them	Restless legs; micro-arousals never noticed
Eye movements in NREM	So-called "Prozac eyes" — REM-like eye movements appearing in NREM	Not noticed by the person, but visible on a sleep study
Bruxism	Teeth grinding is a recognised SSRI effect	Jaw ache, worn teeth, morning headache

The key point

An SSRI can produce, entirely on its own, the exact combination described in Part 5: **vivid dreaming together with unrefreshing sleep**.

The dreams come from REM suppression and rebound plus increased arousals catching those dreams. The flat mornings come from reduced deep sleep and reduced sleep efficiency. One drug, both symptoms.

This does not mean the medication is the cause in any individual case — sleep apnea, narcolepsy, circadian misalignment and nutritional deficiencies produce the same picture, and can coexist with it. It means the medication belongs on the list of possibilities, and that the prescriber needs to know the symptom exists.

A useful clue for the timeline: if the symptoms began or clearly worsened **within weeks of starting the drug or changing the dose**, the medication rises up the list. If they were present long before, or appeared years into stable treatment, something else is more likely to be driving them — though the drug may still be adding to it.

8.6 The liver enzyme story — why fluvoxamine is different

Medicines are broken down mainly by a family of liver enzymes called **cytochrome P450**. Individual members have names like CYP1A2 and CYP2C19. A drug that **inhibits** one of these enzymes slows the clearance of everything else that depends on it — so those other substances build up to higher levels than their dose would suggest.

Fluvoxamine is an unusually broad and potent inhibitor. This is its most clinically important property after its antidepressant effect, and it is the main reason it interacts with more drugs than most SSRIs:

Enzyme	Degree of inhibition	Substances affected
CYP1A2	Very strong — among the most potent known	Caffeine, theophylline, clozapine, olanzapine, duloxetine, some muscle relaxants
CYP2C19	Strong	Diazepam, omeprazole, phenytoin, propranolol, some antidepressants
CYP3A4	Moderate	Many drugs including some statins, some benzodiazepines, certain cardiac drugs
CYP2C9	Moderate	Warfarin, some anti-inflammatories
CYP2D6	Weak	Some antidepressants, some beta blockers, codeine

What this means in practice

Every doctor and every pharmacist you deal with should be told you take fluvoxamine — not only your psychiatrist. That includes a dentist, a physician treating an infection, and anyone prescribing for pain.

It also applies to over-the-counter products and herbal supplements, which people frequently do not think of as medicines. Multi-ingredient herbal sachets are a particularly common source of hidden interactions because the label lists many botanical names and the significant one is easy to miss.

The practical habit: carry a written list, or a photograph of your prescriptions and supplement labels on your phone, and show it rather than reciting it.

8.7 The caffeine interaction in detail

Caffeine is broken down almost entirely by CYP1A2 — the enzyme fluvoxamine inhibits most powerfully. This produces an interaction that is far larger than people expect and almost never connected to the symptom it causes.

	Normally	On fluvoxamine
Caffeine half-life	About 5 hours	Roughly 15–30 hours
Tea at 6pm, by 11pm	About half remaining	Nearly all remaining
Tea at 6pm, by 3am	Largely cleared	Still around half remaining
Effect of daily intake	Clears between days	Accumulates day to day

What this does to sleep is subtle and therefore easily missed. It does not usually stop you falling asleep — fluvoxamine is sedating and often wins that contest. What it does is **block adenosine receptors through the night** (section 3.1), which lightens sleep and increases arousals, particularly in the second half of the night. More arousals means more remembered dreams and a flatter morning, with nothing in the experience pointing back to a cup of tea eleven hours earlier.

A free experiment worth running

Move **all** caffeine — tea included, and tea is frequently overlooked because people think of caffeine as a coffee issue — to before midday. Hold it for **three weeks**, since the accumulated caffeine needs time to clear and sleep takes time to reorganise.

If mornings improve, you have identified a real and freely correctable contributor. If nothing has changed after three weeks, caffeine was not the problem and you can stop wondering about it.

Either result is useful. It costs nothing, carries no risk, and unlike almost everything else in this document it requires no appointment.

Other CYP1A2 points: smoking **induces** CYP1A2, so smokers clear caffeine faster — and someone who stops smoking while on fluvoxamine may suddenly find their usual caffeine intake far more potent. The same applies to any other CYP1A2 drug.

8.8 Melatonin, and other raised drug levels

The same CYP1A2 inhibition affects **melatonin**, which your body produces naturally and which is broken down by that enzyme. On fluvoxamine, natural melatonin levels rise substantially.

Practical consequences:

- **Tell your doctor before taking any melatonin supplement.** Combined with fluvoxamine, blood levels can climb far higher than the dose suggests, which can cause pronounced morning grogginess.
- The same applies to ramelteon, agomelatine and other melatonin-based agents.
- This interaction has actually been used deliberately in research — low-dose fluvoxamine to boost melatonin — which shows how substantial the effect is.

8.9 Sexual side effects

Sexual dysfunction is among the most common effects of SSRIs, and among the most under-reported. Studies that ask patients directly find dramatically higher rates than studies that wait for patients to volunteer the information — a gap that exists because people are reluctant to raise it and doctors often forget to ask.

Mechanism: raised serotonin reduces dopamine activity in pathways governing desire and arousal, and dampens **nitric oxide** signalling — the chemical pathway that produces and maintains an erection. Serotonin also directly inhibits the ejaculatory reflex, which is why SSRIs are sometimes prescribed deliberately for premature ejaculation.

Recognised effects:

- Reduced libido — often the earliest and most noticed.
- Delayed orgasm, or inability to reach orgasm.
- Reduced genital sensation.
- Difficulty achieving an erection, and — distinctly — difficulty **maintaining** one.
- Reduced intensity of orgasm.

The indirect route, which is easy to miss

Because SSRIs **suppress REM sleep**, and nocturnal erections occur exclusively during REM (section 4.7), they can also reduce the frequency and firmness of night-time and morning erections.

This matters for interpretation. Reduced morning erections are normally taken as a sign pointing towards a hormonal or vascular cause. In someone taking an SSRI, part of that reduction may simply be less REM sleep — an entirely different explanation with entirely different implications.

It is worth mentioning the medication explicitly when any doctor asks about morning erections.

These effects are **dose-related and usually reversible**. Recognised management strategies exist — dose reduction, switching to an agent with a lower rate, adding a medication that counteracts the effect, or treating the erectile difficulty directly — but every one of them is a prescribing decision requiring the full clinical picture. The only action available to you is to **report it clearly**, which is genuinely the useful step: it cannot be addressed if it is not known.

8.10 Other side effects, common to rare

Early, usually settling within 2–4 weeks

- Nausea — the commonest reason people stop early. Taking it with food helps.
- Headache, drowsiness, dizziness.
- **Increased anxiety or restlessness in the first week or two.** Counterintuitive but well known, and a reason doses are started low and raised slowly.
- Insomnia or sleep disturbance, despite the sedating effect.

Ongoing

- Sedation and daytime drowsiness.
- Sweating, especially at night.
- Dry mouth, constipation or diarrhoea.
- Appetite and weight change, in either direction.

- **Emotional blunting** — a flattening of both low and high feeling. Some people find this a relief; others find it troubling and describe feeling distant from things they used to care about. It is worth reporting because it is often dose-related.
- Yawning — harmless but common enough to surprise people.
- Bruxism — teeth grinding, which can cause jaw pain and dental damage.

Less common, worth knowing

- **Hyponatraemia** — low blood sodium, more likely in older adults. Symptoms include confusion, headache and unsteadiness.
- **Increased bleeding tendency** — serotonin is involved in platelet function. Relevant before surgery and when combined with anti-inflammatory painkillers or blood thinners.
- Bone density effects with very long-term use.
- QT interval changes on ECG at high doses, or combined with other QT-affecting drugs.

8.11 Serotonin syndrome

Rare but serious. Caused by excessive serotonin activity, usually when serotonergic agents are combined. Onset is typically within hours of the triggering change.

Substances that raise the risk when combined with an SSRI:

- Other antidepressants — particularly MAOIs, which are an absolute contraindication.
- **Tramadol** and **pethidine** — very common in surgical and pain settings, and a frequent real-world trigger.
- Triptans for migraine.
- Linezolid, an antibiotic.
- **St John's Wort** (*Hypericum perforatum*) — a herbal antidepressant that appears in many multi-ingredient supplements. It also induces liver enzymes, altering levels of other medicines. This one is worth specifically checking for on any supplement label.
- Certain cough medicines containing dextromethorphan.
- Recreational stimulants.

Warning signs — needs urgent medical attention:

- Agitation, confusion, restlessness.
- Rapid heart rate, raised blood pressure, fever.
- Heavy sweating, shivering.
- Muscle twitching, tremor, rigidity; overactive reflexes.
- Diarrhoea, nausea; in severe cases seizures.

8.12 Discontinuation and tapering

Stopping an SSRI abruptly causes **discontinuation syndrome**. This is not addiction — there is no craving and no dose escalation — it is the brain readjusting after having adapted to the drug's presence. But it is genuinely unpleasant.

Typical symptoms, usually beginning within a few days:

- Dizziness, unsteadiness.

- "Brain zaps" — brief electric-shock sensations in the head, a distinctive and frequently reported symptom.
- Nausea, flu-like feelings.
- Irritability, anxiety, low mood.
- Insomnia and **vivid dreams or nightmares** — from REM rebound, as explained in section 4.4.

Fluvoxamine's relatively short half-life means discontinuation symptoms tend to appear faster and more prominently than with longer-acting SSRIs. Tapering is therefore gradual — often over weeks to months, with slower steps at the lower doses, where the proportional change is largest.

The one rule that does not bend

Do not reduce, stop, split, skip or substitute this medication on your own — not to test a theory, not to relieve a side effect, not before a medical test, not because a document described an alternative.

Abrupt discontinuation causes withdrawal symptoms and risks the return of the condition being treated, which can be considerably harder to bring back under control than it was to treat the first time.

Every adjustment discussed anywhere in this document is a decision for the prescribing doctor, made with your complete history in front of them. Your job is to bring accurate, specific information to that conversation — which is genuinely the most useful thing a patient can do.

8.13 The six-month review

Six months is a meaningful point. The drug is at full effect, initial side effects have long settled, and whatever remains is likely to persist unless something changes. It is a natural moment for a structured review — not a moment to make changes independently.

What to prepare beforehand

- **Be specific about what improved and what did not.** "Better" is hard to act on. "Mood is steadier but I still ruminate about past events most evenings" is something a doctor can work with.
- **Separate the symptom types.** Intrusive or obsessive thoughts, rumination over the past, low mood, and anxiety are four different things that respond to different treatments. Say which one bothers you most now.
- **List every side effect, including the uncomfortable ones.** Sexual side effects in particular go unmentioned far more often than they go unnoticed.
- **Bring a written sleep record** — two weeks, one line per morning. Template in 9.3.
- **Bring the complete medication and supplement list**, with ingredient labels photographed.
- **Bring recent blood results** — B12, vitamin D, thyroid, liver enzymes, and anything else recent.

Reasonable questions to ask

- "What exactly is this treating in my case?" The answer determines which alternatives exist.
- "Given what is still present, is the current dose right, or should it be adjusted?"
- "Are the side effects I am having likely to come from this drug, and are there options with a different side-effect profile?"
- "Would adding psychological therapy help more than a dose change?" For rumination specifically, structured therapies — CBT, rumination-focused CBT, mindfulness-based cognitive therapy — have good evidence and no sexual side effects.

- "Could this be contributing to my sleep symptoms, and does it affect my sleep testing?"
- "If we ever change or stop it, what would the taper look like?"

Practical Section

9.1 Pattern-matching guide

For orientation only. These patterns overlap heavily and cannot be separated without testing.

Pattern	Points towards
Vivid dreams + tired mornings + snoring or witnessed breathing pauses + nocturia	Obstructive sleep apnea — overnight study
Vivid dreams + tired mornings beginning within weeks of a medication start or dose change	Medication effect on sleep architecture — discuss with prescriber
Short naps genuinely refreshing + sleepiness that persists even when nights improve	Central hypersomnia including narcolepsy — MSLT
Long naps that are not refreshing + severe morning grogginess + long night sleep	Idiopathic hypersomnia
Sleepy at the wrong times but fine when allowed to sleep on own schedule	Circadian rhythm disorder — especially after years of shift work
Tired all day, no clear sleep pattern, abnormal blood results	Nutritional / metabolic — B12, vitamin D, iron, thyroid, testosterone
Sleepiness + sudden muscle weakness triggered by emotion or laughter	Cataplexy — strongly suggests type 1 narcolepsy. Mention explicitly
Leg discomfort at rest relieved by movement; partner reports kicking	Restless legs / periodic limb movements — check ferritin
Acting out dreams physically	REM sleep behaviour disorder — needs assessment

9.2 Sleep hygiene that actually has evidence

None of these replace investigation. None carry risk. Several are genuinely effective, and they are listed roughly in order of impact.

- **Consistent wake time, seven days a week.** The single most powerful habit on this list. Wake time anchors the body clock far more strongly than bedtime does. Sleeping in at weekends undoes the week's progress.
- **Morning daylight, 15–30 minutes within an hour of waking.** The strongest available signal for resetting a clock that has drifted late. Outdoor light, even on a cloudy day, is many times brighter than indoor lighting.
- **Caffeine before midday only.** And on fluvoxamine this matters considerably more than usual (section 8.7). Remember tea, cola, chocolate and some painkillers also contain it.
- **Naps under 30 minutes, before 3pm** (section 6.2) — unless a doctor has advised scheduled naps as treatment.
- **Get out of bed if awake more than about 20 minutes.** Lying awake trains the brain to associate the bed with wakefulness. Go elsewhere, do something dull in dim light, return when sleepy. This is a core component of CBT for insomnia, which outperforms sleeping tablets in the long term.
- **Bedroom dark, cool and quiet.** Cool matters physiologically — falling core temperature is a sleep trigger (section 3.4).

- **Screens dimmed and away in the final hour.** The issue is melatonin suppression by light, not eye strain.
- **A warm shower or bath 1–2 hours before bed.** Counterintuitive but effective — it accelerates the drop in core temperature afterwards.
- **No alcohol as a sleep aid.** It shortens sleep onset but suppresses REM, fragments the second half of the night and worsens apnea. A common and costly mistake.
- **Regular exercise, not in the two hours before bed.**
- **Correct what is measurably low** — B12, vitamin D, iron, thyroid. Cheap and they compound with everything else.
- **Write down tomorrow's worries before bed.** A short written list measurably reduces bedtime rumination — a small intervention with surprisingly good evidence.

9.3 A sleep diary template you can copy

Two weeks of this is worth more than months of recollection, and it is the single most useful thing to bring to either doctor. One line each morning, thirty seconds:

Date	Bed	Asleep approx	Wakes	Up	Rested /10	Dreams	Naps	Day sleepy /10	Notes
14/9	11:15	11:45	2	06:30	4	Yes, vivid	30 min 4pm	7	Tea at 6pm
15/9	11:00	11:20	1	06:30	6	No	None	5	No evening tea

The two columns that matter most are **Rested /10** and **Day sleepy /10**. Tracking them separately reveals whether they move together or independently — which, as section 6.6 explained, is one of the most diagnostically useful observations available. Add any change you make (caffeine timing, dose change, illness) in Notes.

9.4 Questions to take to the psychiatrist

- What is this medication treating in my case, specifically?
- Is the current dose right given what is still present?
- Which of my symptoms could be side effects rather than the illness?
- Could this medication be affecting my sleep quality or my dreams?
- Could it be contributing to sexual side effects, and what are the options?
- Would psychological therapy add anything that a dose change would not?
- Is there anything in my other medicines or supplements that interacts with this?
- If a sleep study requires adjusting this medication, is that safe for me?
- What blood tests would you recommend given ongoing fatigue?

9.5 Questions to take to the sleep physician

- Here is my complete medication and supplement list — does anything need adjusting before testing?
- Will REM-suppressing medication affect the validity of my MSLT?

- How should I prepare in the two weeks beforehand?
- Will the overnight study also check for limb movements and oxygen levels?
- When the report comes, can you walk me through the AHI, the stage percentages, the arousal index, the mean sleep latency and the SOREMP count?
- If the results are normal, what is the next step rather than the end of the road?

9.6 Red flags that need prompt attention

- **Falling asleep while driving**, or near-misses from sleepiness. Stop driving and seek help urgently — this is the most dangerous symptom in this whole document.
- **Witnessed breathing stopping** during sleep.
- **Acting out dreams physically**, especially with injury risk.
- **Sudden muscle weakness while awake**, triggered by emotion.
- **Signs of serotonin syndrome** (section 8.11) — particularly after starting any new medicine or supplement.
- **Any significant worsening of mood**, or thoughts of harming yourself. Contact your doctor or a crisis service immediately — this is always urgent, and always worth raising even if it feels difficult to say.

Glossary

Term	Meaning
Adenosine	Chemical that builds up while awake and creates sleep pressure. Blocked by caffeine.
AHI	Apnea-Hypopnea Index — breathing disturbances per hour. Under 5 normal, 5–15 mild, 15–30 moderate, over 30 severe.
Alpha waves	8–12 Hz brain waves of relaxed wakefulness with eyes closed; disappear at sleep onset.
Apnea	Complete stop in airflow lasting at least 10 seconds.
Arousal index	Brief arousals per hour of sleep. Under 15 is normal.
Atonia	Muscle paralysis normally accompanying REM, preventing you acting out dreams.
Bruxism	Teeth grinding during sleep. A recognised SSRI side effect.
Cataplexy	Sudden loss of muscle tone while fully awake, triggered by emotion. Specific to type 1 narcolepsy.
Chronotype	Where your body clock naturally sits — early type, late type, or in between. Largely genetic.
Circadian rhythm	The body's roughly 24-hour internal clock. Process C.
CPAP	Continuous Positive Airway Pressure — the standard treatment for obstructive sleep apnea.
CYP450	Family of liver enzymes that break down drugs. Fluvoxamine inhibits several strongly.
Delta waves	Under 4 Hz. The large slow waves of deep N3 sleep.
Discontinuation syndrome	Symptoms from stopping an antidepressant too quickly. Not addiction, but a reason to taper.
Glymphatic system	The brain's waste-clearing network, most active during deep sleep.
Hypnagogic hallucination	Dream imagery experienced while falling asleep. Can feel real and frightening.
Hypnic jerk	Sudden muscle twitch as you fall asleep. Normal and very common.
Hypnogram	Graph of sleep stages across the night. The most informative page of a sleep report.
Hypopnea	Partial reduction in airflow with an oxygen drop or an arousal.
Idiopathic hypersomnia	Excessive sleepiness with long unrefreshing naps and severe sleep inertia. Distinct from narcolepsy.
K-complex	Large single wave in N2, often the brain suppressing an arousal.
Melatonin	Hormone signalling biological night. A timing signal, not a sedative. Suppressed by light.
MSLT	Multiple Sleep Latency Test — daytime nap study measuring sleep latency and SOREMPs.
N1, N2, N3	The three NREM stages: light, intermediate, and deep slow-wave sleep.
Nocturia	Waking to urinate at night. Commonly caused by sleep apnea and frequently misattributed.
Nocturnal penile tumescence	Automatic erections during REM sleep, three to five per night.
NREM	Non-Rapid Eye Movement sleep — the quiet, physically restorative family of stages.
Orexin	Also called hypocretin. Brain chemical stabilising wakefulness. Lost in type 1 narcolepsy.
Parasomnia	Unwanted event during sleep — sleepwalking, night terrors, nightmares, sleep paralysis.
PLMD	Periodic Limb Movement Disorder — repetitive leg movements causing arousals. Linked to low iron.
Polysomnography	The full overnight sleep study.

Term	Meaning
Process C	The circadian component of sleep regulation — when you should sleep.
Process S	The homeostatic component — how much sleep pressure has built up.
REM	Rapid Eye Movement sleep — the vividly dreaming stage, body paralysed, brain highly active.
REM latency	Time from sleep onset to first REM. Normally 70–100 minutes.
REM rebound	Intensified REM following a period of REM suppression. A common cause of vivid dreams.
Reuptake	A nerve cell reabsorbing a chemical messenger it released. Blocked by SSRIs.
SCN	Suprachiasmatic nucleus — the cluster of brain cells that is the master body clock.
Sigma-1 receptor	A receptor fluvoxamine activates, unusually among SSRIs.
Sleep architecture	The overall structure of a night — how much of each stage, in what order.
Sleep debt	Accumulated sleep loss. Partly repayable; deep sleep recovers first, REM second.
Sleep efficiency	Percentage of time in bed actually spent asleep. Above 85% is good.
Sleep inertia	Grogginess after waking. Worst when woken out of deep N3 sleep.
Sleep onset latency	Minutes from lights out to falling asleep. Normally 10–20.
Sleep paralysis	Waking while REM atonia is still active. Frightening but harmless.
Sleep spindle	Burst of fast activity in N2. Involved in memory filing and blocking noise.
Social jetlag	Mismatch between your body clock and your imposed schedule.
SOREMP	Sleep Onset REM Period — REM within 15 minutes of sleep onset. Two or more on MSLT is a key narcolepsy finding.
SSRI	Selective Serotonin Reuptake Inhibitor — the drug class fluvoxamine belongs to.
Serotonin syndrome	Dangerous excess of serotonin activity, usually from combining serotonergic drugs.
Theta waves	4–8 Hz. Characteristic of N1 and N2.
WASO	Wake After Sleep Onset — total minutes awake during the night after first falling asleep.

Closing note

This guide covers general medical science, not your individual case. Sleep disorders, mood disorders and medication effects overlap in ways that require proper testing and a doctor who knows your full history.

Take it with you. Mark what applies. Ask the questions. And do not change any medication without the prescriber's agreement.

Understanding Sleep, Dreams and Fluvoxamine

A plain-language guide to sleep stages, REM, vivid dreaming, daytime sleepiness, and how Obsace ER (fluvoxamine) affects all of them

How to read this guide

This is written to be understood without any medical background. Every technical term is explained the first time it appears, and there is a glossary at the end.

Parts 1 and 2 explain how normal sleep works. Part 3 and Part 4 explain what it means when sleep goes wrong in the two specific ways described here — too many vivid dreams with no morning energy, and short daytime naps that feel more restorative than a whole night. Part 5 covers fluvoxamine in detail. Part 6 is a practical checklist.

Important

This document explains general medical concepts. It is not a diagnosis and not a treatment plan, and it cannot replace the doctors who know your history.

Nothing here should be used to start, stop, or change the dose of any prescribed medicine. Fluvoxamine and bupropion both require supervised changes — stopping either suddenly can cause withdrawal symptoms and a return of the illness being treated.

The most useful way to use this guide is to read it, mark the parts that sound like you, and take those questions to your psychiatrist and sleep physician.

Part 1 — What Sleep Actually Is

Most people picture sleep as a switch: you are awake, then you are off, then you are awake again. That picture is wrong, and almost every confusing thing about sleep becomes clear once you replace it.

Sleep is not one state. It is a **sequence of several different states**, each doing a different job, cycling through the night in a repeating pattern. Think of it less like switching off a light and more like a night shift in a factory: different teams clock in at different hours, each doing work the others cannot do. If one team never gets its shift, the factory opens in the morning with part of the work undone — even though the building was closed all night.

That is the core idea. **Time in bed is not the same as work completed.** You can sleep eight hours and still wake up with the job half done.

The two families of sleep

Everything divides into two families:

- **NREM sleep** — Non-Rapid Eye Movement. This is the quiet, physically restorative kind. It has three stages, numbered N1, N2 and N3.
- **REM sleep** — Rapid Eye Movement. This is the active, dreaming kind. Your brain is almost as busy as when you are awake, but your body is deliberately paralysed.

In a healthy adult night, roughly 75–80% of sleep is NREM and 20–25% is REM.

Stage N1 — the doorway

Length: 1–7 minutes. **Share of the night:** about 5%.

This is the thin border between wakefulness and sleep. Your muscles loosen, your thoughts start drifting into disconnected images, and your breathing slows. If someone wakes you in N1, you will usually insist you were never asleep — and you will half be right.

This is also the stage where **hypnic jerks** happen: that sudden kick of a leg or arm, sometimes with a feeling of falling. It is a normal glitch as the brain hands over control of the muscles, and almost everyone gets them.

Stage N2 — the workhorse

Length: 10–25 minutes per cycle, lengthening through the night. **Share of the night:** about 45–55% — more than any other stage.

This is where you spend roughly half your sleeping life. Body temperature drops, heart rate slows, and the brain produces two distinctive electrical signatures: **sleep spindles** (short bursts of fast activity) and **K-complexes** (single large waves).

Those spindles are not decoration. They are the brain filing the day's learning into long-term storage, and they also work as a shield — they help keep outside noise from waking you. When someone says they are a light sleeper, part of what they are describing is weak spindle activity.

Stage N3 — deep sleep, the repair shift

Length: 20–40 minutes in early cycles, shrinking to almost nothing by morning. **Share of the night:** about 15–25%.

Also called slow-wave sleep or deep sleep. The brain's electrical activity slows into large, rolling waves. This is the hardest stage to be woken from — if someone shakes you awake out of N3 you will be confused, slow and disoriented for several minutes.

This is the stage that makes you feel *physically* rested. During N3:

- Growth hormone is released in its largest pulse of the day — tissue repair, muscle recovery.
- The immune system does much of its maintenance work.
- The brain's waste-clearing system runs at its highest rate, flushing out metabolic by-products that build up during waking hours.
- Most of the body's testosterone production happens during sleep, concentrated in these deeper stages.

Critical detail: N3 is front-loaded. The large majority of your deep sleep happens in the first half of the night. This is why a person who sleeps from 11pm to 3am and then has a broken second half still gets most of their physical restoration — and why someone whose *first* few hours are disturbed suffers much more than the clock would suggest.

REM sleep — the dreaming shift

Length: 10 minutes in the first cycle, up to 45–60 minutes by the last. **Share of the night:** about 20–25%.

REM is the strangest state the human body produces. Measured at the scalp, your brain looks almost awake — fast, active, energetic. Your eyes dart rapidly behind closed lids. Breathing and heart rate become irregular. And yet your body cannot move at all.

REM atonia — why you cannot move

During REM, the brainstem actively switches off the signals going to your voluntary muscles. You are temporarily paralysed from the neck down, apart from the eyes and the diaphragm.

This is protective. Your brain is generating a full experience — running, fighting, falling — and without atonia you would physically act it out and injure yourself.

Two familiar experiences come straight from this system misfiring. **Sleep paralysis** is waking up while the atonia is still switched on, so your mind is awake but your body will not move. **REM behaviour disorder** is the opposite — the atonia fails and the person physically acts out their dream.

REM also does work you cannot do without:

- **Emotional processing.** REM appears to strip the emotional charge from difficult memories while keeping the memory itself. It is sometimes described as overnight therapy.
- **Creative and associative memory.** NREM files facts; REM connects them to each other.
- **Nocturnal erections.** In men, erections occur automatically during REM — typically three to five per night, totalling well over an hour. This is why morning erections exist: you have simply woken up at the end of a REM period.

How the stages fit together across a night

One full pass through the stages is a **sleep cycle**, and it takes roughly 90 minutes — anywhere from 70 to 120. A normal night contains four to six of them.

The order within a cycle is approximately: N1 → N2 → N3 → back up to N2 → REM → brief near-waking → start again. That brief surfacing between cycles is normal and you almost never remember it.

But — and this is the part most people never learn — **the cycles are not identical**. The mix shifts steadily through the night:

Cycle	Roughly when	Deep sleep (N3)	REM sleep
1st	First 90 min	Large — 20 to 40 minutes	Very short — 5 to 10 minutes
2nd	90–180 min	Still substantial	Longer — 15 to 20 minutes
3rd	3–4.5 hours in	Shrinking fast	Growing — 20 to 30 minutes
4th	4.5–6 hours in	Often almost none	Long — 30 to 45 minutes
5th / 6th	Final hours	Essentially none	Longest — up to 60 minutes

The single most useful fact in this guide

Deep sleep dominates the first half of the night. REM dominates the second half.

Almost everything that follows — why cutting sleep short costs you REM specifically, why morning dreams are the vivid ones, why a drug that suppresses REM changes the shape of your whole night — comes from this one asymmetry.

A practical consequence: if you normally sleep seven hours and one night you sleep five, you do not lose two hours evenly spread. You lose almost entirely REM, because REM is what was scheduled for those last two hours.

Part 2 — Dreams: Why They Are Vivid, and Why You Remember Them

Dreaming and remembering a dream are two completely different things. Confusing them is the source of nearly every misunderstanding people have about their own dreams.

Where dreams come from

Dreams occur in both REM and NREM, but they are not the same kind of experience:

- **REM dreams** are vivid, visual, emotional, story-shaped and often bizarre. They have characters, plots, sudden shifts of scene. When people say "I had a dream," this is what they mean.
- **NREM dreams** are thin — more like a thought or a static image than a film. "I was thinking about work" rather than "I was being chased through a building that was also my school."

In REM, the brain regions handling vision and emotion are running at full intensity, while the prefrontal cortex — the part responsible for logic, sequence and asking "wait, does this make sense?" — is largely offline. That combination is exactly why dreams are richly detailed and simultaneously absurd, and why the absurdity only becomes obvious after you wake.

Why you only remember some dreams

You dream every single night, four to six times, whether you recall it or not. Nobody stops dreaming. What changes is **recall**, and recall depends almost entirely on one thing:

The rule of dream recall

You remember a dream only if you wake up during it, or within a couple of minutes of it ending.

Dream content is held in an extremely fragile short-term store. If you pass smoothly from REM into the next NREM stage without surfacing, the content is simply never transferred to long-term memory, and by morning it is gone as though it never happened.

This produces a conclusion that catches most people by surprise: **remembering many vivid dreams is usually not a sign of more dreaming. It is a sign of more waking.**

Someone who reports "I dream all night, every night, and they are so vivid" is very often describing a night that is being repeatedly interrupted — dozens of brief arousals they are not consciously aware of, each one catching a dream in progress and stamping it into memory. The dreams were always there. What is new is the fragmentation.

REM rebound — pressure that builds up

REM behaves like a debt. If you are deprived of it — by short sleep, by alcohol, by certain medicines — the pressure accumulates, and the moment the obstacle is removed the brain takes it back with interest. More REM than usual, entered faster than usual, and unusually intense.

This is **REM rebound**, and it is the mechanism behind several things people find alarming: the flood of strange dreams after a few sleep-deprived nights, the intense dreaming some people get when they stop drinking alcohol, and — importantly for Part 5 — the vivid dreaming that antidepressants

can cause.

What makes dreams more vivid

Cause	What it does
Fragmented sleep (any cause)	More awakenings during REM, so far more dreams get remembered
Sleep apnea	Breathing pauses cause arousals, many of them out of REM
Antidepressants	Suppress REM, then REM rebounds harder and more intensely
Alcohol	Suppresses REM in the first half; strong rebound in the second half
Withdrawal from alcohol or sedatives	Large REM rebound — often the most vivid dreaming people experience
Sleeping later than usual	More time spent in the REM-rich final hours
Fever, stress, grief	Increased arousals plus heavier emotional processing load
Narcolepsy	REM intrudes where it does not belong, including at sleep onset

Part 3 — Vivid Dreams Plus No Energy on Waking: What It Means

This is one of the most common sleep complaints there is, and also one of the most informative, because the two halves of it point in the same direction.

The short version

Heavy dream recall plus unrefreshing mornings is, in most cases, a description of fragmented sleep.

The dreams are remembered because the night is being broken up. The mornings are flat because the same fragmentation is preventing the deep, consolidated sleep that produces the feeling of restoration. One underlying problem, two visible symptoms.

The person has spent eight hours in bed. The *clock* says they slept. But the sleep was shallow and repeatedly interrupted, so the restorative work never got done.

Why fragmentation is invisible to the person it happens to

An arousal does not have to be a full awakening. The brain can lift out of deep sleep into light sleep for three to fifteen seconds — enough to abort the restorative process — and then drop back down, leaving no memory whatsoever.

Someone with moderate sleep apnea may have **two to four hundred** of these in a night and sincerely report that they sleep straight through without waking. They are not wrong about their experience. They are simply unable to perceive it. This is the single biggest reason sleep problems need measuring rather than self-reporting.

The realistic causes, and how each one feels

1. Medication effects

Antidepressants — SSRIs in particular — suppress REM and increase brief arousals. The result is exactly this pattern: intense dreaming plus unrefreshing sleep. Covered fully in Part 5.

2. Obstructive sleep apnea

The throat's soft tissue collapses during sleep, blocking airflow. Oxygen drops, the brain briefly arouses to reopen the airway, breathing resumes, sleep returns — and this repeats all night. Classic signs: loud snoring, witnessed pauses in breathing, waking with a dry mouth or headache, and daytime sleepiness. It is more likely with excess weight, a thick neck, or nasal obstruction — but it occurs in slim people too, which is why it is frequently missed. Only an overnight study can detect it.

3. Circadian misalignment

Your body has an internal clock that decides *when* you should be asleep. If that clock is shifted later than your working life demands, you spend your night sleeping at biologically wrong hours and your morning waking during your internal night. The sleep may be structurally fine; it is happening at the wrong time. Years of rotating shift work is a common cause, and the misalignment can persist long

after the shifts stop.

4. Periodic limb movements

Repetitive leg movements during sleep, every 20 to 40 seconds, each potentially causing a micro-arousal. Sufferers rarely know. A bed partner usually does.

5. Central hypersomnias, including narcolepsy

Here the sleepiness is generated primarily rather than caused by poor-quality nights. Discussed in Part 4, because the nap pattern is the clue that separates it from everything above.

6. Nutritional and metabolic contributors

Low vitamin B12, low vitamin D, low iron or ferritin, and thyroid disorders all cause fatigue and can disturb sleep. Low iron and low B12 are specifically linked to restless legs. These are worth correcting regardless of what else is found, because they are simple and they overlap with every other cause on this list.

An important caution about self-assessment

Every cause above produces broadly the same felt experience: tired mornings, poor sleep quality, heavy dreaming. You cannot tell them apart from the inside, and neither can a doctor from the description alone. That is not a failure of attention — it is the nature of the problem, and it is precisely why sleep studies exist.

Part 4 — Why a Short Daytime Nap Can Beat a Whole Night

This is the detail people most often dismiss as strange or embarrassing, and it is in fact one of the most diagnostically valuable things a person can report. There are four explanations, and they point to very different places.

Explanation 1: Sleep inertia

Sleep inertia is the grogginess after waking. It is a real physiological state — reduced blood flow to parts of the brain, which takes time to normalise. It typically lasts 15 to 30 minutes but after disrupted sleep it can stretch to an hour or more.

Its severity depends almost entirely on **which stage you were in when you woke**. Waking out of deep N3 sleep produces heavy, disabling inertia. Waking out of light N2 or REM produces very little.

Here is why that matters for naps. A short nap of 20 to 30 minutes usually stays in N1 and N2 and never reaches deep sleep — so you wake cleanly and feel sharp. A whole night ends differently, and if the night has been disturbed the brain may be catching up on deep sleep at the very moment your alarm goes off. You wake straight out of N3, and feel terrible. The night gave you more sleep; the nap gave you a better *exit*.

Explanation 2: The afternoon circadian dip

Your internal clock does not produce one sleepy period per day. It produces two. There is a strong drive to sleep at night, and a second, weaker one in the early-to-mid afternoon — roughly 1pm to 4pm for most people. This is biological, not a consequence of lunch.

Sleeping through that dip works with the clock rather than against it, so the sleep is efficient and the waking is clean. It is the same mechanism behind the siesta in many cultures.

Explanation 3: The nap is simply better sleep

If the underlying problem is worse at night — apnea that is worse lying flat for hours, or a body clock misaligned with the sleeping hours — then an afternoon nap may genuinely be higher-quality sleep. Less time to accumulate disruption, possibly a different position, different timing. Two hours of unbroken sleep can beat eight fragmented ones.

Explanation 4: Narcolepsy

This is the one that matters most to identify, because it is a specific diagnosis with specific treatment.

What narcolepsy actually is

Narcolepsy is not "falling asleep suddenly" as films portray it. It is a disorder of the brain's control over the boundary between sleeping and waking.

In type 1 narcolepsy, the brain has lost the cells producing **orexin** (also called hypocretin) — a chemical that holds the awake state stable. Without enough of it, the boundaries blur in both directions: sleep leaks into the day, and wakefulness leaks into the night.

That two-way leak explains a picture that otherwise seems contradictory: **overwhelming daytime sleepiness together with a broken, restless night**. People are often surprised that a condition of excessive sleep involves sleeping badly.

Features that point towards narcolepsy:

- **Refreshing short naps.** A 15–20 minute nap produces genuine, noticeable restoration — often for a couple of hours. This is considered fairly characteristic; in most other causes of sleepiness, short naps help much less.
- **Persistent daytime sleepiness that does not track night-time quality.** Improving the nights does not fix the days, because the daytime drive is being generated independently.
- **Vivid dreaming and dreaming during naps.** REM appearing where it should not — including within minutes of falling asleep, when normally it takes 70 to 100 minutes.
- **Sleep paralysis and hypnagogic hallucinations** — dream imagery intruding as you fall asleep or wake, sometimes frightening and mistaken for something psychiatric.
- **Cataplexy** — sudden loss of muscle tone while fully awake, triggered by emotion, classically laughter. It may be as subtle as the jaw slackening or the knees buckling. When present it is near-diagnostic; it is absent in type 2 narcolepsy.

How the two tests distinguish these

Polysomnography is the overnight study. Sensors record brain waves, eye movements, muscle activity, breathing, oxygen levels, heart rhythm and leg movements. It is what produces the **AHI** — apnea-hypopnea index, the number of breathing disturbances per hour. Under 5 is normal; 5–15 mild; 15–30 moderate; above 30 severe. It also measures how much time was spent in each stage and how many arousals occurred — the objective version of "I slept badly."

MSLT — the Multiple Sleep Latency Test — runs the next day. You are given four or five scheduled 20-minute nap opportunities, roughly two hours apart, in a dark quiet room. Two things are measured:

- **Mean sleep latency** — average minutes to fall asleep across the naps. Eight minutes or less indicates pathological daytime sleepiness.
- **SOREMPs** — Sleep Onset REM Periods. Naps in which REM appears within 15 minutes. Normally REM should not appear in a short nap at all. **Two or more SOREMPs, together with a short mean latency, is the core diagnostic finding for narcolepsy.**

This is why the overnight study must come first: it confirms you slept adequately the night before, so the next day's sleepiness cannot be dismissed as simple sleep deprivation.

Why medication timing matters enormously for the MSLT

The MSLT diagnoses narcolepsy by detecting REM where it does not belong. Antidepressants — SSRIs, SNRIs and tricyclics — **suppress REM**. That is their well-documented effect on sleep architecture.

If REM is being pharmacologically suppressed on the day of the test, SOREMPs may not appear even in someone who genuinely has narcolepsy. The result can come back falsely normal.

For this reason, standard protocols generally require REM-suppressing medication to be stopped roughly **two weeks** before testing — though whether that is safe depends entirely on the psychiatric condition being treated, and it is a decision for the treating doctors, not the patient.

There is a second trap on the other side: **abruptly stopping** an antidepressant causes REM rebound, which can create SOREMPs in someone who does not have narcolepsy — a false positive. Both directions of error are real, which is why a properly planned taper matters and why the sleep physician needs the complete medication list.

Part 5 — Fluvoxamine (Obsace ER) in Detail

Obsace ER is a brand of **fluvoxamine**, an SSRI — Selective Serotonin Reuptake Inhibitor. It was the first SSRI approved for obsessive-compulsive disorder, and in India it is prescribed most often for OCD, intrusive thoughts and anxiety disorders, sometimes for depression. The ER means extended release: the dose is delivered gradually rather than all at once, which smooths out side effects and allows once-daily dosing, usually at bedtime.

How it works, in plain terms

Nerve cells communicate by releasing chemical messengers into the gap between them. Serotonin is one such messenger. After it has delivered its signal, the releasing cell pulls it back in and recycles it — this is **reuptake**.

Fluvoxamine blocks that recycling pump. Serotonin therefore stays in the gap longer and keeps signalling. Over weeks, the brain adapts to this sustained increase, and it is the adaptation — not the immediate chemical change — that produces the therapeutic effect. This is why these drugs take four to six weeks to work properly, and up to ten to twelve weeks for OCD symptoms specifically.

Standard dosing

	Fluvoxamine ER
Usual starting dose	100 mg once daily, at bedtime
Usual therapeutic range	100 – 300 mg daily
Maximum	300 mg daily
Typical OCD dose	Often towards the higher end of the range
Time to full effect	4–6 weeks for mood; 10–12 weeks for OCD symptoms

A dose of 100 mg therefore sits at the *lower* end of the range. That is neither good nor bad in itself — it depends on what is being treated and how well it is working. It simply means there is room to move if the treating doctor judges that appropriate.

Effect on sleep architecture

This is the part most relevant to everything in Parts 1 to 4. SSRIs change the shape of sleep in consistent, documented ways:

Effect	What happens	What it feels like
REM suppression	Total REM time falls; time to first REM lengthens	Fewer dreams initially; less emotional processing
REM rebound	Suppressed REM returns with increased intensity	Unusually vivid, strange, memorable dreams
Increased arousals	More brief awakenings through the night	Sleep feels shallow; more dreams recalled
Reduced sleep efficiency	Less of the time in bed is actual sleep	Eight hours in bed feels like five
Reduced deep sleep	N3 slow-wave sleep can decrease	Waking unrefreshed despite adequate hours
Periodic limb movements	Can be induced or worsened by SSRIs	Restless legs; micro-arousals you never notice

So an SSRI can produce, on its own, the exact combination described in Part 3: vivid dreaming together with unrefreshing sleep. Fluvoxamine at bedtime concentrates this effect on the night, which is usually the right trade because it is sedating — but it means the sleep-architecture effects land squarely on your sleeping hours.

The caffeine interaction — unique to fluvoxamine

Why this matters more than people expect

Caffeine is broken down by a liver enzyme called **CYP1A2**. Fluvoxamine is one of the most powerful known inhibitors of that enzyme — far stronger in this respect than other SSRIs.

Normally caffeine has a half-life of around five hours. On fluvoxamine, that can extend to roughly **15 to 30 hours**.

In practical terms: a cup of tea at 6pm may still be working at half strength at 2 or 3am. It will not usually stop you falling asleep. What it does is lighten the sleep and increase arousals through the second half of the night — producing more remembered dreams and a flat morning, with no obvious connection in the person's mind back to the tea.

The same enzyme block raises the body's own **melatonin** levels substantially. Anyone on fluvoxamine should mention it before taking any melatonin supplement.

This interaction is worth testing rather than assuming. Moving all caffeine — tea included — to before midday for three weeks is free, carries no risk, and if mornings improve you have identified a real contributor. If nothing changes after three weeks, the tea was not the problem and you can stop wondering.

Sexual side effects

Sexual dysfunction is among the most common side effects of SSRIs, and among the most under-reported — studies that ask directly find far higher rates than studies that wait for patients to volunteer it.

The mechanism: increased serotonin reduces dopamine activity and dampens nitric oxide signalling, which is the chemical pathway that produces and sustains an erection. Common effects include reduced desire, delayed or absent orgasm, reduced genital sensitivity, and difficulty achieving or *maintaining* an erection.

Because SSRIs also reduce REM sleep, and nocturnal erections occur during REM, they can additionally reduce the frequency and firmness of morning erections — which is worth knowing before interpreting that as a sign of something else.

These effects are dose-related and usually reversible. There are recognised strategies — dose reduction, switching agents, adding a medicine that counteracts it — but every one of them is a prescribing decision.

Other side effects worth knowing

- **Early weeks:** nausea, headache, drowsiness, increased anxiety. Usually settle within two to four weeks.
- **Ongoing:** sedation, sweating, dry mouth, appetite change, emotional blunting — a flattening of both low and high feeling that some people find helpful and others find troubling.
- **Drug interactions:** fluvoxamine inhibits several liver enzymes, so it raises blood levels of many other medicines — including caffeine, theophylline, some tricyclic antidepressants, clozapine, warfarin and certain benzodiazepines. Every doctor you see should be told you are taking it, and

so should any pharmacist.

- **Serotonin syndrome:** rare but serious. Risk rises when combined with other serotonergic agents — including tramadol, triptans, linezolid, and the herbal remedy St John's Wort (*Hypericum perforatum*), which appears in many multi-ingredient supplements without being obvious on the label.
- **Discontinuation syndrome:** stopping abruptly can cause dizziness, electric-shock sensations, nausea, irritability, insomnia and vivid dreams. This is not addiction — it is the brain readjusting — but it is the reason tapering must be gradual and supervised.

If you have been taking it for around six months

Six months is a meaningful point. The drug is at full effect, initial side effects have long settled, and whatever remains is likely to persist unless something changes. It is a natural moment for a structured review — not a moment to make changes independently.

What to prepare before the appointment

- **Be specific about what has improved and what has not.** "Better" is hard to act on. "Mood is steadier but I am still ruminating about past events most evenings" is something a doctor can work with.
- **Separate the symptom types.** Intrusive or obsessive thoughts, rumination over the past, low mood, and anxiety are four different things that respond differently. Which one is bothering you most now?
- **List every side effect, including the uncomfortable ones.** Sexual side effects in particular go unmentioned far more often than they go unnoticed, and a doctor cannot address what they are not told.
- **Bring a written sleep record.** Two weeks of one line per morning — bedtime, wake time, how rested out of 10, dreams yes or no, naps taken — is worth more than months of recollection.
- **Bring the full medication list.** Every prescription, supplement and herbal product, with the ingredient labels where possible. Multi-ingredient supplements are a frequent source of hidden interactions.
- **Bring recent blood results.** B12, vitamin D, thyroid, liver enzymes.

Reasonable questions to ask

- "What exactly is this treating in my case — depression, OCD, anxiety?" The answer determines which alternatives even exist.
- "Given the symptoms still present, is the current dose right, or should it be adjusted?"
- "Are the side effects I am having likely to be from this drug, and are there options with a different side-effect profile?"
- "Would adding psychological therapy help more than a dose change?" For rumination specifically, evidence for structured therapy — CBT, rumination-focused CBT, mindfulness-based cognitive therapy — is strong, and it carries no sexual side effects.
- "Could this medication be contributing to my sleep symptoms, and does it affect my sleep testing?"
- "If we ever change or stop it, what does the taper look like?"

The one rule that does not bend

Do not reduce, stop, split, skip or substitute this medication on your own — not to test a theory, not to improve a side effect, not before a medical test.

Abrupt discontinuation causes withdrawal symptoms and risks the return of the condition being treated. Every adjustment discussed in this document is a decision for the doctor prescribing it, made with your full history in front of them.

Your job is to bring accurate, specific information to that conversation. That is genuinely the most useful thing a patient can do.

Part 6 — Putting It Together

A rough guide to what different patterns suggest

This is for orientation only. These patterns overlap heavily and cannot be separated without testing.

Pattern	Points towards
Vivid dreams + tired mornings + loud snoring or witnessed breathing pauses	Obstructive sleep apnea — overnight study needed
Vivid dreams + tired mornings, starting after a medication began or changed	Medication effect on sleep architecture — discuss with prescriber
Short naps genuinely refreshing + sleepiness that persists even when nights improve	Central hypersomnia including narcolepsy — MSLT needed
Sleepy at the wrong times, fine if allowed to sleep on own schedule	Circadian rhythm disorder — especially after years of shift work
Tired all day with no clear sleep pattern, plus abnormal blood results	Nutritional or metabolic cause — B12, vitamin D, iron, thyroid
Sleepiness plus sudden muscle weakness triggered by emotion or laughter	Cataplexy — strongly suggests type 1 narcolepsy; mention explicitly

Things that help regardless of the diagnosis

None of these replace investigation, but none of them carry risk either, and several are genuinely effective:

- **Consistent wake time**, seven days a week including weekends. The wake time anchors the body clock more powerfully than bedtime does.
- **Morning daylight**, 15 to 30 minutes within an hour of waking. This is the strongest available signal for resetting a misaligned clock.
- **Caffeine before midday only** — and on fluvoxamine, this matters considerably more than usual.
- **Cap naps at 20–30 minutes, before 3pm**. Longer naps reach deep sleep, causing heavy sleep inertia on waking, and they reduce the sleep pressure needed for a good night.
- **Screens dimmed and put away** in the hour before bed; keep the bedroom dark, cool and quiet.
- **Correct what is measurably low** — B12, vitamin D, iron. Simple, cheap, and they compound with everything else.
- **Regular exercise**, but not in the two hours before bed.
- **Keep a written sleep diary**. Memory for sleep is unreliable; two weeks of notes changes what a doctor can see.

A final word on how to use all this

The purpose of understanding sleep in this much detail is not to reach your own diagnosis. It is the opposite — the detail should make clear *why* self-diagnosis does not work here. Every cause in

Part 3 produces the same subjective experience, and the things that distinguish them — an AHI number, a mean sleep latency, a count of SOREMPs, a testosterone level — are all invisible from the inside.

What this knowledge does give you is the ability to notice the right things, describe them precisely, ask informed questions, and understand the answers. That is a real advantage. Patients who can say "short naps refresh me but full nights do not, and that has not changed even though my nights improved" get better care than patients who can only say "I am tired."

Glossary

Term	Meaning
AHI	Apnea-Hypopnea Index — breathing disturbances per hour of sleep. Under 5 normal, 5–15 mild, 15–30 moderate, over 30 severe.
Atonia	The muscle paralysis that normally accompanies REM sleep, preventing you from acting out dreams.
Cataplexy	Sudden loss of muscle tone while fully awake, usually triggered by strong emotion. Specific to type 1 narcolepsy.
Circadian rhythm	The body's roughly 24-hour internal clock, which decides when you should be asleep or alert.
CYP1A2	A liver enzyme that breaks down caffeine and several medicines. Strongly inhibited by fluvoxamine.
Discontinuation syndrome	Symptoms caused by stopping an antidepressant too quickly. Not addiction, but a reason to taper.
Hypnagogic hallucination	Dream imagery experienced while falling asleep; can feel real and frightening.
Hypnic jerk	The sudden muscle twitch as you fall asleep. Normal and very common.
MSLT	Multiple Sleep Latency Test — daytime nap study measuring how fast you fall asleep and whether REM appears.
N1, N2, N3	The three stages of NREM sleep: light, intermediate, and deep (slow-wave) respectively.
NREM	Non-Rapid Eye Movement sleep — the quiet, physically restorative family of sleep stages.
Orexin	Also called hypocretin. A brain chemical that stabilises wakefulness. Lost in type 1 narcolepsy.
Polysomnography	The full overnight sleep study, recording brain waves, breathing, oxygen, heart and limb movement.
REM	Rapid Eye Movement sleep — the active, vividly dreaming stage, with the body paralysed.
REM rebound	The intensified REM that follows a period of REM suppression. A common cause of unusually vivid dreams.
Reuptake	The process by which a nerve cell reabsorbs a chemical messenger it has released. Blocked by SSRIs.
Sleep architecture	The overall structure of a night's sleep — how much of each stage, in what order.
Sleep efficiency	The percentage of time in bed actually spent asleep. Above 85% is generally considered good.
Sleep inertia	The grogginess after waking. Worst when woken out of deep N3 sleep.
Sleep spindle	A burst of fast brain activity in N2 sleep, involved in memory consolidation and blocking noise.

Term	Meaning
SOREMP	Sleep Onset REM Period — REM appearing within 15 minutes of falling asleep. Two or more on MSLT is a key narcolepsy finding.
SSRI	Selective Serotonin Reuptake Inhibitor — the class of antidepressant that fluvoxamine belongs to.

Closing note

This guide covers general medical science, not your individual case. Sleep disorders, mood disorders and medication effects overlap in ways that require proper testing and a doctor who knows your full history.

Take it with you. Mark what applies. Ask the questions. And do not change any medication without the prescriber's agreement.